Cash Transfers, Mental Health, and Agency: Evidence from an RCT in Germany *

Sandra Bohmann, Susann Fiedler, Maximilian Kasy,
Jürgen Schupp, Frederik Schwerter†

March 22, 2026

Abstract

Mental health and wellbeing are unequally distributed in high-income countries, disadvantaging low-income individuals. Unconditional, regular, guaranteed, and individual cash transfers may help address this inequality by promoting financial security and agency. We conducted a preregistered RCT in Germany, where treated participants received monthly payments of EUR 1,200 for three years. Cash transfers improve mental health and wellbeing substantially, with effects persisting 18 months after the program ended. Cash transfers also improve perceived autonomy, savings, prosocial giving, time with friends, and sleep. Our findings suggest that cash transfers improve mental health and wellbeing if they empower agency and life changes.

JEL Codes: C93, I31, D10
Keywords: Basic Income; RCT; Purpose in Life; Life Satisfaction

*The RCT in this paper received ethics approval by the University of Cologne (Reference Number 20008FS) and was pre-registered in the AEA RCT Registry at https://www.socialscienceregistry.org/trials/7734. We thank Mein Grundeinkommen for the implementation of the RCT. Bohmann: LMU München; sandra.bohmann@lmu.de. Fiedler: WU Wien; susann.fiedler@wu.ac.at. Kasy: Oxford University; maxkasy@gmail.com. Schupp: DIW and FU Berlin; jschupp@diw.de. Schwerter: Frankfurt School of Finance and Management, CESifo, IZA; f.schwerter@fs.de. †Corresponding author.

1 Introduction

Good mental health and wellbeing enable individuals to be resilient, realize their abilities, learn well, work productively, contribute to their communities and live healthier lives (Prince et al., 2007; WHO, 2021). Even in high-income countries, however, mental health and wellbeing outcomes are unequally distributed: economically disadvantaged individuals tend to have poorer mental health and wellbeing (Fryers et al., 2003; Marmot, 2013). Unconditional, regular, guaranteed, and individual cash transfers are a promising policy tool to address this inequality (Pateman, 2004; Van Parijs and Vanderborght, 2017; Marinescu, 2018; Bidadanure, 2019). Such cash transfers reduce uncertainty about future income, and empower agency of recipients. Greater financial security and agency may allow individuals to better fulfill personal needs and pursue their own values. In addition, greater financial security and autonomy directly benefit mental health and wellbeing (Ryan and Deci, 2017; Frey et al., 2004; Bartling et al., 2014; Guan et al., 2022).

Randomized controlled trials (RCTs) in low- and middle-income countries indeed find that cash transfers can lead to improvements in mental health and wellbeing (Baird et al., 2013; Haushofer and Shapiro, 2016; Christian et al., 2019; McGuire et al., 2022). However, contemporaneous cash-transfer RCTs suggest that such improvements do not extend to high-income countries (West and Castro, 2023; Jaroszewicz et al., 2024; Miller et al., 2024; Balakrishnan et al., 2024). One explanation, consistent with classical insights from psychology (Ryan and Deci, 2001; Diener and Biswas-Diener, 2002), is that cash transfers may be less impactful in high-income countries because basic needs are already met and material aspirations grow as income rises. Alternatively, previous cash-transfer programs in high-income countries may not have been generous enough to promote financial security and agency.

In this paper, we present evidence from a generous cash transfer program that we implemented in Germany in the form of a preregistered RCT. The treatment group received unconditional, regular, guaranteed, and individual cash transfers of EUR 1,200 on a monthly basis for a total of three years. Cash transfers improve mental health and wellbeing during the study period. These improvements remained roughly constant over the duration of the cash transfer program and persisted up to 18 months after the final transfer. In additional analyses, we find that cash transfers empower recipients to make meaningful changes in their lives: Treated participants save more, give more to others, they spend more time with friends, and sleep longer and better. In addition, cash transfers improved perceived autonomy of recipients. Overall, our findings suggest that cash transfer programs can lead to lasting improvements of mental health and wellbeing if they are regular, guaranteed, unconditional, individual, and generous enough to empower agency.

In Section 2, we describe the design of our RCT. Our goal is to estimate the causal effect of unconditional, regular, guaranteed, and individual cash transfers on mental health and wellbeing. We restricted eligibility to single-person households with below-average income, which has two important implications. First, it ensures treatment representativeness: each recipient has full individual control over the transfer, without having to negotiate its use within a shared household budget, as envisioned in leading proposals for cash transfers (Pateman, 2004; Van Parijs and Vanderborght, 2017). From an implementation perspective, this restriction also yields independent survey responses across participants, avoiding the possibility that members of the same household may influence each other's reports. Second, our study sample is also policy-relevant. Single-person households constitute roughly one third of all households in the United States and Europe, and the literature identifies living alone as a structural risk factor for loneliness and, in turn, for the onset of depression (Wu et al., 2022; Ma et al., 2022; Holt-Lunstad, 2024). Combined with our participants' below-average baseline income, this makes our sample a population at elevated risk for poor mental health and wellbeing.

We estimate causal effects using a stratified random assignment design. We used participants' responses to a baseline survey for the stratification. Members of the treatment group received monthly payments of EUR 1,200 for a total of three years. Cash transfers increased their baseline monthly household income by 46% to 110%. After the baseline survey, we measured self-reported mental health and wellbeing of the treatment and control group in eight follow-up surveys. Waves 1 through 6 were administered during the cash transfer program at six-month intervals; waves 7 and 8 were administered 6 and 18 months after the final transfer.

We measure mental health using the WHO-5 Wellbeing Index (Staehr, 1998) and five items from the Perceived Stress Scale (PSS-10, Cohen et al.; Cohen, 1983; 1988). The WHO-5 WBI has high clinimetric validity and is a sensitive and specific screener for depression (Topp et al., 2015). The PSS questions are widely used to measure perceived stress (Yılmaz Koğar and Koğar, 2024). Both instruments correlate with physical health outcomes and health-related behaviors (Topp et al., 2015; Richardson et al., 2012; Tenk et al., 2018). In addition, we measure participants' eudaimonic and evaluative wellbeing. To this end, we adapted questions from the German Socio-Economic Panel (SOEP) on participants' perceived purpose in life and participants' general life satisfaction, and their satisfaction in several life domains (Richter et al., 2017).

In Section 3, we present the main results of our RCT. We first present treatment effects averaged across all survey waves during the three-year program. Unconditional, regular, and guaranteed cash transfers improve mental health by 0.306 SD, purpose in life by 0.250 SD, and life satisfaction by 0.426 SD during the study period. These treatment effects are statistically significant based on robust standard errors, and on Neyman and Fisher's exact p-values that account for our stratified random treatment assignment (Athey and Imbens, 2017). The effect on mental health is present in survey instruments. The effect on life satisfaction is present for general life satisfaction and for the domains of income, health, sleep, and leisure. We also find directionally positive but statistically insignificant improvements of work satisfaction and satisfaction with social life.

Second, we consider the dynamics of treatment effects during the cash transfer program. The effects are mostly constant in time. There are two important exceptions: (i) The improvement in income satisfaction decreases over time. (ii) While we find no average effect on work satisfaction over the full study period, we find a delayed positive effect during the final 18 months of the program.

Third, we study the temporal persistence of treatment effects 6 and 18 months after the final cash transfer. To quantify this, we aggregate our mental health, purpose in life, and life satisfaction into a single mental health and wellbeing (MHW) index. This MHW index improved by 0.379 SD during the program compared to the control, by 0.331 SD 6 months after the program, and by 0.362 SD 18 months after the program, implying that on average 91% of the during-program treatment effect persisted.

In Section 4, we show in exploratory analyses that cash transfers empower participants to implement and experience life changes that are meaningful for mental health and wellbeing. In particular, we document five patterns: (i) Recipients of cash transfers state that they feel greater financial security, and their self-reported household finance data suggests that they saved one third of their monthly cash transfers for the future. (ii) Treated participants shared seven to eight percent of their cash transfers with others, in the form of financial support to family and friends and charitable giving. (iii) Cash transfers allowed recipients to spend 1.3 hours more time per week with their friends. (iv) Treated participants tended to sleep longer (between 1.2 and 2.3 hours per week) and invested more in recreational activities. (v) Cash transfers improved the perceived autonomy of recipients.

Importantly, there is ample evidence that mental health and wellbeing benefit from greater financial security (Jachimowicz et al., 2022; Guan et al., 2022), prosocial behavior (Dunn et al., 2008; Hui et al., 2020), social connectedness (Harandi et al., 2017; Park et al., 2020; Waldinger and Schulz, 2023; Annan and Archibong, 2023), sleep and recreational activities (Scott et al., 2021; Kuykendall et al., 2015), and autonomy (Inglehart et al., 2008; Steckermeier, 2021). These five patterns provide suggestive evidence for the mechanisms driving the main treatment effects. Cash transfers improve mental health and wellbeing directly by allowing recipients to experience greater financial security and agency and indirectly by allowing them to implement meaningful life changes.

In addition, we study the labor supply effects of the cash transfers of our RCT in a companion paper (Bernhard et al., 2025). There, we find no reduction in recipients' labor force participation. This likely supports the positive treatment effects on mental health and wellbeing in this paper. Employment provides time structure, social contact, collective purpose, status, and activity that are essential for mental health and wellbeing independently of income (Jahoda, 1981, 1982; Paul et al., 2023).

In Section 5, we compare our results to the related literature. The most comparable study is Miller et al. (2024); Vivalt et al. (2024); Bartik et al. (2024), who study monthly cash transfers of USD 1.000 for a total of three years in the US and *rule out* mental health improvements beyond 0.028 SD (Miller et al., 2024). A key difference is that their transfers accrued to households with, on average, three members, likely diluting both the effective generosity and the individual discretionary control over the transfer. We argue that individual-level transfers are more likely to empower the kind of personally meaningful life changes that drive lasting improvements in mental health and wellbeing, and that differences in the institutional context, including Germany's largely universal health insurance and more comprehensive social safety net, may also contribute to the divergent findings. Notably, the study sample in Miller et al. (2024) was more economically disadvantaged than ours, yet we find large effects where they find none—the opposite of what standard diminishing-returns reasoning would predict, but consistent with individual control over the transfer being the operative margin.

We also contribute to the literature that estimates the causal effect of money on mental health and wellbeing more broadly. Such estimates are mainly based on lottery winnings and provide a mixed picture: mental health is found to be affected positively (Gardner and Oswald, 2007; Apouey and Clark, 2015), not at all (Lindqvist et al., 2020) or, potentially even, negatively (Raschke, 2019); life satisfaction is reported to be affected positively (Apouey and Clark, 2015; Lindqvist et al., 2020; Dwyer and Dunn, 2022), or not at all (Kuhn et al., 2011). We argue that unconditional, regular, guaranteed, and individual cash transfers are particularly suited to improve mental health and wellbeing. Their recurring nature limits hedonic adaptation, while their predictable monthly structure integrates into recipients' normal budgeting routines rather than requiring active financial management of a lump sum.

In Section 6, we discuss our findings in light of internal and external validity concerns. Regarding internal validity, we show that our results are robust to multiple-hypothesis-testing adjustments and argue that alternative explanations do not provide reasonable accounts for our findings. In particular, experimenter demand effects, social desirability concerns, Hawthorne effects, a disappointed control, and differential attrition are unlikely to be the drivers of our results (as supported by additional analyses). Regarding external validity, we discuss limitations of the generalizability of our results.

We conclude in Section 7 and discuss policy implications of our findings.

2 Design and Analyses

We estimate the causal effect of a generous, regular, guaranteed, unconditional, and individual cash-transfer program on recipients' mental health and wellbeing in a preregistered, randomized controlled trial in Germany.[1] We display an overview of the timeline of the RCT in Figure 1. In the following, we describe the setup of the RCT in more detail and discuss the analyses that we use to evaluate the RCT.

[1] We study the labor supply effects of our RCT in a companion paper (Bernhard et al., 2025). The description of the RCT in the companion paper hence overlaps in parts with the description of the RCT below.

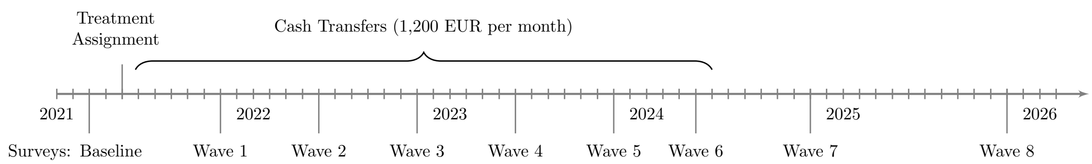

Figure 1: Timeline of the randomized controlled trial. The treatment group (N=107) received unconditional monthly cash transfers of EUR 1,200 from Spring 2021 through Spring 2024, for a total of three years. Both treated and control participants (N=1,580) completed a baseline survey prior to treatment assignment and eight follow-up surveys. Waves 1 through 6 were administered during the cash transfer program at six-month intervals; waves 7 and 8 were administered 6 and 18 months after the final transfer. The cash transfer program, survey schedule, and treatment assignment are indicated along the timeline.

2.1 Study sample and treatment assignment

The German NGO Mein Grundeinkommen e.V. (MG) financed the RCT, including the cash transfers. MG is funded through private donations. Prior to the RCT, MG made regular cash transfers of EUR 1,000 per month for a single year to 818 randomly assigned applicants, making MG a credible partner to finance the cash transfers in our RCT.[2]

We used a multi-step sampling and treatment assignment procedure to construct our study sample. The steps in this procedure are (i) a public call and voluntary registration of potential participants, (ii) selection of a subsample based on demographic and economic eligibility criteria, (iii) stratified sampling of eligible registrants to construct a representative baseline sample, members of which were then invited to fill out a baseline survey, (iv) blocking of participants in the baseline sample who completed the baseline survey, based on a rich set of baseline covariates, and random assignment to treatment within blocks, and (v) selection of a representative subsample of blocks based on the budget constraints of the study.

Public call On August 18, 2020, MG and the German Institute for Economic Research (DIW Berlin) publicly announced the launch of the RCT during Spring/Summer 2021 and made a public call to apply for participation in the RCT. The announcement included a description of the main features of the study: Selected participants of the study would be randomly assigned to a treatment group or a control group; treatment and control groups would participate in semi-annual online surveys; members of the treatment group would receive monthly payments of 1,200.00 EUR for three years; members of the control group would receive monetary incentives to complete the surveys; additional research activities may be offered. During signup, we collected the following screening information: Age, gender, education, monthly net income, number of people living in their household, number of kids, zip code, and their general attitude towards universal basic income. Between August 18 and December 10 in 2020, 2,048,370 potential participants registered in response to this public signup call.

[2]We have no data on these single-year cash transfers and hence cannot evaluate them in this paper.

Study sample goals and eligibility criteria Achieving a representative study sample while providing sufficiently generous and genuinely individual cash transfers was financially infeasible. We hence prioritized treatment representativeness and restricted our sample to single-person households. This ensured that each recipient retains full discretionary control over the transfer, as envisioned in leading proposals for unconditional cash transfers (Pateman, 2004; Van Parijs and Vanderborghť, 2017), and allows us to obtain independent survey responses across participants. Within these constraints, we selected a subsample of registered individuals (called "baseline sample") based on the following additional eligibility criteria with respect to participants' demographic and socioeconomic characteristics.

(i) Participants have to be between 21 and 40 years old.

(ii) Participants are required to be German residents and to have a monthly net income between 1,100.00 (slightly below the poverty line in 2021) and 2,600.00 EUR

(iii) Individuals who, at the time of the baseline survey, were receiving social benefits for long-term unemployment are excluded from participation.[3]

∗ Participants whose status on any of the criteria (including household size) changed after treatment assignment stayed in the study sample.[4]

Eligibility criteria (i)-(iii) ensured comparability to contemporaneous cash-transfer studies (Miller et al., 2024; Vivalt et al., 2024; Bartik et al., 2024) and (iii) also implied that our cash transfers would not reduce potential long-term unemployment benefits of recipients. By focusing on individual households, our cash transfers were relatively more generous than in other cash-transfer studies (Miller et al., 2024; Vivalt et al., 2024; Bartik et al., 2024).

Baseline survey and stratification Among the potential participants who satisfied these criteria, our implementation partner invited 20,000 individuals to our baseline survey (see Appendix On.1). We considered 8,971 of these participants, who completed the baseline, in the randomized block assignment. Amongst the 11,029 individuals we did not consider, 49% were no longer eligible and/or had missing responses in the baseline variables used for stratification, 33% did not complete the baseline survey, and 17% did not sign the data sharing consent form.

We use the answers during signup and to the baseline survey to sort participants into homogenous blocks. In particular, we use questions on mental health, wellbeing, gender, socio-economic background, financial household, and political attitudes. We display the full list of stratification variables used for blocking in Appendix On.2.

[3]Given current benefit eligibility rules, such social benefits would have been cut by up to the full amount of the cash transfer if these individuals were to participate in our study. The net transfer to such individuals would thus have been significantly below the expenditure for MG.

[4]By wave 6, approximately one third of participants in both the treatment and control group lived in households with more than one member, and roughly 6% had children. These rates are balanced across treatment and control, suggesting that household status changes are not differentially driven by the cash transfers.

We construct blocks that minimize the total sum of Mahalanobis distances in the stratification variables between pairs of observations within blocks. We chose the number of participants per block to be 32 because of the following constraints and considerations: (i) Our implementation partner was willing to finance 107 treated participants and 1589 control participants.(ii) Within each block, we assign two participants to the treatment group to be able to calculate standard errors for the sample average treatment effect (see Athey and Imbens, 2017, and our discussion of inference below), but not more than two participants to keep the number of blocks as large as possible and each block as homogenous as possible.

This procedure results in 281 blocks, while our project budget allows for 53 blocks. We select the final set of blocks in two steps. First, we discard all blocks with a maximum within-block distance greater than 14 (to avoid poorly matched observations), as well as one block with less than 32 observations. Second, amongst the remaining 273 blocks, we selected a representative sample of blocks that matched the distribution of gender, education groups, and income groups of eligible participants in the screening survey.

Final sample Within each of the remaining 53 blocks, we randomly assigned two participants to the treatment group. Because the funding allowed for an odd number of treated participants, one additional individual from one block was chosen at random to participate in the treatment. After the selected participants in the treatment group and control group were informed about their treatment status, seven individuals in the control group wanted to be excluded from the study sample, one individual in the treatment group resigned his/her spot in the treatment group because of a job opportunity outside of Germany, and one individual in the treatment group could not be reached. For each of these two missing individuals, we sampled one individual from the control group within the same block, to receive the corresponding treatment status. Since only two participants were replaced, we do not adjust our inference for this reassignment.

Our final study sample hence includes 107 participants in the treatment group and 1,580 participants in the control group. Treatment and control group are well-balanced in stratification variables, see Table 1.

2.2 Treatment conditions and outcomes

Cash transfers Members of the treatment group received cash transfers of EUR 1,200, paid monthly, over the course of three years. Importantly, the cash transfers were tax-free, see Appendix On.3. Cash transfers increased the baseline monthly household income of the treatment group by 46% to 110%. There were no conditions attached to receiving the cash transfers, apart from completing six semi-annual surveys. All surveys refer to online surveys that lasted between 15 and 20 minutes and were distributed to participants by a professional German online survey company. Members of the control group did not receive cash transfers, and were asked to complete the same six semi-annual surveys. For every completed survey, control participants received an incentive of EUR 10, plus an additional payment of EUR 30 if they completed all

Outcome	Treated	Control	Difference	SE	t-statistic	p-value
Age 29-32	0.355	0.331	0.024	0.048	0.498	0.619
Age 33-40	0.336	0.373	-0.036	0.048	-0.757	0.449
Female	0.477	0.412	0.065	0.050	1.290	0.197
German citizen	0.981	0.981	0.000	0.014	0.014	0.989
UBI proponent	0.505	0.547	-0.042	0.050	-0.837	0.403
Tenure	0.766	0.766	0.000	0.043	0.005	0.996
Education: Lower	0.037	0.038	0.000	0.019	-0.020	0.984
Education: Intermediate	0.215	0.214	0.001	0.041	0.035	0.972
Education: Vocational	0.243	0.241	0.002	0.043	0.044	0.965
Education: Academic	0.037	0.054	-0.016	0.019	-0.843	0.399
Net monthly income	1944.888	1925.767	19.121	40.181	0.476	0.634
Monthly saving	271.607	296.407	-24.800	24.742	-1.002	0.316
Wealth	25327.103	25392.157	-65.054	4450.093	-0.015	0.988
Debt	10170.374	9077.122	1093.252	2655.173	0.412	0.681
High financial security	0.327	0.312	0.016	0.047	0.329	0.742
Working for money	0.935	0.944	-0.010	0.025	-0.383	0.702
In training or education	0.178	0.151	0.027	0.038	0.691	0.489
In vocational training	0.411	0.432	-0.021	0.050	-0.421	0.674
Searching work	0.037	0.038	0.000	0.019	-0.020	0.984
Sick days	7.776	10.850	-3.075	1.152	-2.669	0.008
Weekly hours worked	37.826	37.346	0.480	1.458	0.329	0.742
Political preferences (PC1)	0.015	0.142	-0.127	0.142	-0.893	0.372
Political preferences (PC2)	0.164	0.053	0.112	0.125	0.893	0.372
Subjective wellbeing (PC1)	-0.360	-0.129	-0.231	0.183	-1.263	0.207
Body mass index	24.656	25.452	-0.797	0.490	-1.627	0.104
Transfers to others	363.551	330.733	32.819	103.753	0.316	0.752
Donations in 2020	100.664	96.562	4.101	21.002	0.195	0.845
Binary gender	1.000	1.000	0.000	0.000	–	–

Table 1: Balance of baseline covariates between the treatment group (N=107) and the control group (N=1,580): We report means for the treatment and control groups, the difference in means, heteroskedasticity-robust standard errors, t-statistics, and p-values. All statistics are computed without adjustment for the blocked treatment assignment ("naïve" inference). The covariates listed correspond to the stratification variables used in the blocked random assignment described in Section 2.1. Income and financial variables are denominated in EUR. Covariates are well balanced between the two groups; the only p-value below 0.05 is for sick days, and this difference is not statistically significant after adjusting for multiple comparisons. Education categories refer to the highest school-leaving qualification obtained within the German secondary school system: Lower secondary (Hauptschulabschluss); Intermediate secondary (Realschulabschluss); Upper secondary vocational (Fachhochschulreife) denotes a qualification granting access to universities of applied sciences; Upper secondary academic (Abitur) denotes the general university entrance qualification after 12–13 years.

six surveys. We incentivized survey participation of our control group to limit attrition, which we discuss in detail below. To study the persistence of treatment effects after the end of the cash transfer program, we invited all participants to a seventh and eighth survey six and 18 months after the final payment, and paid treated and control participants who completed the survey EUR 20.

Mental health and wellbeing outcomes To construct our outcomes, we use all questions on participants' mental health and wellbeing from the baseline survey. All of these questions were also included in the follow-up waves.[5] Specifically, we use participants' responses to these questions in the baseline survey ($t = 0$), the survey waves during the cash transfer program ($1 \leq t \leq 6$), and the final survey waves six ($t = 7$) and 18 ($t = 8$) months after the program. The wording of these questions is stated in Appendix On.4.

Our mental health outcomes are based on the WHO-5 questionnaire (Staehr, 1998) and a subset of the PSS-10 questionnaire (Cohen et al., 1983). The WHO-5 questionnaire has high clinimetric validity and is a sensitive and specific screener for depression (Topp et al., 2015). The PSS questions are widely used to measure perceived stress (Yılmaz Koğar and Koğar, 2024). Both indicators correlate with physical health outcomes and health-related behaviors (Topp et al., 2015; Richardson et al., 2012; Tenk et al., 2018). The endpoints of the Likert response scales are "0" and "5" for the WHO-5 questions and "1" and "5" for the PSS questions. After inverting negatively framed PSS questions, larger item values imply better mental health for each question of both instruments. We then obtain wave-specific scores for both instruments (dep_{it} and str_{it}) by summing the respective item values for each participant i and non-missing survey wave t.

Our wellbeing outcomes are based on a single-item question that elicits participants' perceived meaningfulness of their life (eudaimonic wellbeing) and on seven single-item questions that elicit separate aspects of participants' evaluative wellbeing: their general life satisfaction and their health, sleep, work, income, leisure, and social satisfaction. The endpoints of the response scales for these questions are "1" and "11." Larger response values imply greater purpose in life or satisfaction, respectively. We obtain scores for purpose in life (p_{it}), general life satisfaction (g_{it}), health satisfaction (h_{it}), sleep satisfaction (s_{it}), income satisfaction (i_{it}), work satisfaction (w_{it}), social satisfaction (o_{it}), and leisure satisfaction (l_{it}) for each participant i and survey wave t based on the corresponding response values.

We construct our outcomes based on these mental health, purpose in life and life satisfaction scores. In doing so, we consider the mental health, purpose in life and life satisfaction scores both individually for each wave $t > 0$ and averaged on the participant-level across all waves during the cash transfers program, $0 < t < 7$. We then subtract from each of these scores its corresponding baseline score and divide by the study sample's standard deviation of the baseline score, see Table On.7 in Appendix On.6. Our outcomes are standardized changes relative to

[5] The WHO-5 questions and the general life satisfaction question were missing in wave $t = 1$, and the PSS questions were missing in wave $t = 2$.

the baseline. This allows us to estimate treatment effects that are adjusted for treatment-control imbalances at baseline, which remained despite stratification. Moreover, this allows us to directly compare our findings to the related literature that primarily presents treatment effects in standard deviations. We also present the results based on outcomes in levels, as we discuss below.

2.3 Analyses

Our analyses follow three preregistered steps. First, we estimate treatment effects by considering the strata-level difference in mean outcomes between the treatment group and control group, averaged across strata. Second, we determine statistical significance of treatment effects based on robust standard errors, and Neyman and Fisher's exact p-values (Athey and Imbens, 2017) that account for stratified assignment. Third, we apply the Benjamini-Hochberg procedure (Benjamini and Hochberg, 1995) to adjust for multiple hypothesis testing.

Object of interest We denote individual treatment assignment by D and outcomes by Y. Our primary object of interest is the sample average treatment effect

$$\delta = \frac{1}{n} \sum_i (Y_i^1 - Y_i^0), \tag{1}$$

for the individual-level outcomes Y_i for individuals i and corresponding potential outcomes Y_i^0, Y_i^1. Our primary estimator is based on block-level differences in mean outcomes, averaged across blocks b:

$$\begin{aligned} \bar{Y}_b^1 &= \frac{1}{n_b^1} \sum_{i:\, b_i = b} D_i Y_i & \qquad \bar{Y}_b^0 &= \frac{1}{n_b^0} \sum_{i:\, b_i = b} (1 - D_i) Y_i \\ \hat{\delta}_b &= \bar{Y}_b^1 - \bar{Y}_b^0 & \qquad \hat{\delta} &= \frac{1}{N} \sum \hat{\delta}_b, \end{aligned} \tag{2}$$

where n_b^1 and n_b^0 are the number of treated and untreated individuals in block b, and N is the number of blocks.

Inference Inference is based on two alternative methods, both of which yield valid inference for the sample average treatment effect: (i) Standard errors and confidence intervals based on a normal approximation, and (ii) randomization inference.

To calculate a standard error for $\hat{\delta}$ as an estimator of δ, we calculate block-level standard-errors (allowing for arbitrary heteroskedasticity), and aggregate:

$$\begin{aligned} \hat{\sigma}_b^{2,1} &= \frac{1}{n_b^1 - 1} \sum_{i:\, b_i = b} D_i \cdot (Y_i - \bar{Y}_b^d)^2 & \qquad \hat{\sigma}_b^{2,0} &= \frac{1}{n_b^0 - 1} \sum_{i:\, b_i = b} (1 - D_i) \cdot (Y_i - \bar{Y}_b^d)^2 \\ \hat{\sigma}_b^2 &= \frac{1}{n_b^1} \hat{\sigma}_b^{2,1} + \frac{1}{n_b^0} \hat{\sigma}_b^{2,0} & \qquad \hat{\sigma}^2 &= \frac{1}{N} \sum_b \hat{\sigma}_b^2. \end{aligned} \tag{3}$$

95% confidence intervals for δ are then calculated as

$$CI = [\hat{\delta} - 1.96 \cdot \hat{\sigma}, \hat{\delta} + 1.96 \cdot \hat{\sigma}]. \tag{4}$$

Neyman p-values (denoted p-N in our tables) are similarly based on these standard errors and the normal approximation for the distribution of $\hat{\delta}$.

Our second approach toward inference is based on permutations of treatments, that is, based on randomization inference. This approach allows us to test the null hypothesis that the intervention had no effect of any kind, that is, $Y_i^1 = Y_i^0$ for all individuals i and potential outcomes Y_i^1, Y_i^0. We re-assign treatment at random *within* each of the blocks b. For this counterfactual treatment assignment, we re-calculate any given test-statistic. Repeating this process many times, we calculate the share of re-assignments for which the test-statistic is bigger than the realized value of the test-statistic. This share is the Fisher p-value (denoted p-F in our tables) for the null hypothesis of no effects.

Compound hypotheses In order to deal with the issue of multiple testing in a principled manner, we preregistered to use the Benjamini–Hochberg procedure, which allows us to control the false discovery rate, that is, the share of rejected hypotheses which in fact hold true. This procedure works as follows. We sort the p-values for each of the m hypotheses tested by size, resulting in ordered values $P_{(j)}$. For a critical value α, we find the largest value k such that

$$P_{(k)} \leq \frac{k}{m}\alpha. \tag{5}$$

We reject the null hypothesis for all $i = 1, \ldots, k$.

We preregistered to apply this procedure separately for different groups of outcomes and specified mental health and wellbeing as one group of outcomes.

2.4 Preregistration

We preregistered the RCT at the AEA registry (https://www.socialscienceregistry.org/trials/7734). We originally preregistered that each block consists of two treated participants, 26 control participants, and 4 "spare participants," because our implementation partner was initially only willing to finance survey participation for 26 control participants per block. Our implementation partner thereafter changed their willingness and we were able to add the spare participants to the control group, resulting in 1,580 rather than 1,377 control participants; see Section 2.1 for details.

In this paper, we analyze all measures included in the baseline survey that capture mental health and wellbeing constructs. The baseline survey was finalized before treatment assignment; we provide a list of all items from the baseline questionnaire (excluding labor supply items, which are analyzed in Bernhard et al., 2025) as a supplementary document (Online Appendix). We did not include these survey-based measures in the preregistration. The preregistration, however,

Outcome	Treated	Control	ATE	SE	t-stat	p-N	p-F	n treated	n control
Aggregates									
MHW Index	0.305	-0.074	0.379	0.077	4.912	0.000	0.000	107	1477
Mental Health	0.322	0.016	0.306	0.074	4.122	0.000	0.002	107	1477
Purpose in Life	0.116	-0.134	0.250	0.087	2.888	0.004	0.004	107	1476
Life Satisfaction	0.312	-0.114	0.426	0.075	5.657	0.000	0.000	107	1477
Aggregate components									
WHO-5 Depression	0.355	0.036	0.319	0.079	4.066	0.000	0.000	107	1445
PSS Stress	0.261	-0.034	0.295	0.080	3.706	0.000	0.002	107	1470
General Life Satisfaction	0.275	-0.076	0.351	0.080	4.377	0.000	0.000	107	1445
Domain Satisfaction Index	0.311	-0.113	0.424	0.087	4.894	0.000	0.000	107	1477
Domain satisfactions									
Health Satisfaction	-0.017	-0.308	0.291	0.088	3.315	0.001	0.002	107	1477
Sleep Satisfaction	0.191	-0.099	0.290	0.088	3.290	0.001	0.000	107	1477
Work Satisfaction	-0.046	-0.189	0.143	0.096	1.484	0.138	0.142	107	1471
Income Satisfaction	0.540	-0.011	0.551	0.108	5.099	0.000	0.000	107	1477
Leisure Satisfaction	0.408	0.163	0.245	0.092	2.663	0.008	0.006	107	1476
Social Satisfaction	0.116	-0.009	0.125	0.072	1.739	0.082	0.104	107	1476

Table 2: Average treatment effects on mental health and wellbeing outcomes during the three-year cash transfer program (wave 1-6): We report average treatment effects (ATE) in standard deviations on our outcomes as changes relative to the baseline, see outcome definitions in Table On.7. Inference is based on robust standard errors (SE), and Neyman (N) and Fisher's exact (F) p-values. We reject the null of no effect for the aggregated mental health, purpose in life, and life satisfaction outcomes, the WHO-5 depression scale, the PSS stress scale, the domain satisfaction index, general life satisfaction, and health, sleep, income, and leisure satisfaction. We cannot reject the null for work and social satisfaction. All treatment effects that are statistically significant based on unadjusted p-values remain significant after applying the Benjamini-Hochberg procedure to control the false discovery rate across the m = 56 mental health and wellbeing hypotheses tested in Tables 2, Table A.1, Table A.2, and Table A.3, with the exception of sleep satisfaction 6 months after the program (Table A.2).

included the collection of hair samples to measure cortisol levels as a biomarker for stress. We were unable to collect a sufficient number of valid samples to permit any meaningful analyses due to low participation in hair sample collection.

3 Main results

To obtain estimates for treatment effects during the cash transfer program, we estimate average treatment effects on outcomes across all waves one through six (see the right-most column in Table On.7). We show these results in Table 2. Our mental health and wellbeing (MHW) index improves by 0.379 standard deviations, mental health by 0.306 SD, purpose in life by 0.250 SD, and life satisfaction by 0.426 SD. Improvements in mental health are separately present for the WHO-5 depression screener (0.319 SD) and the PSS questions (0.295 SD). Improvements are also separately present for general life satisfaction (0.351 SD) and the domain satisfaction index (0.424 SD). Income satisfaction improves by 0.551 SD, health satisfaction by 0.291 SD, sleep satisfaction by 0.290 SD, and leisure satisfaction by 0.245 SD.

All of these improvements correspond to statistically significant treatment effects based on Neyman and Fisher's exact p-values, see Table 2. Social and work satisfaction also improve, by 0.125 SD and 0.143 SD respectively, but these treatment effects are not statistically significant. For work satisfaction, the average of treatment effects over time masks a delayed effect during the final three waves, as we discuss below.

We also report average outcomes separately for treated participants and control participants in Table 2. Treatment effects for the mental health outcomes, income satisfaction, leisure satisfaction, and social satisfaction are mainly the result of greater positive changes relative to the baseline for treated participants than for control participants. Treatment effects for general life satisfaction, sleep satisfaction, and purpose in life are the result of both positive changes for treated participants and negative changes for control participants. Health and work satisfaction depreciated on average relative to the baseline both for treated and control participants, but depreciation was less strong for treated participants.

Robustness: outcomes in levels We show in Table A.1 in Appendix A.1 that our results are robust to considering outcomes in levels, that is, without adjustment for treatment-control imbalances at baseline, which remained despite stratification. There are two differences, however: The treatment effect on purpose in life in levels is only weakly statistically significant and the treatment effect on social satisfaction is statistically significant. These different results reflect small and statistically insignificant treatment-control imbalances at baseline for purpose in life (-0.132 SD) and social satisfaction (0.055 SD).

Dynamics during the cash transfer program We display treatment effects separately for all waves and outcomes in Figures 2 and 3. As treatment effects are calculated by adjusting for baseline imbalances in outcomes, there are, by construction, no outcome differences at baseline. Figures 2 and 3 show that the improvements remain fairly constant over time during the cash transfer program, with two notable exceptions: (i) The improvement in income satisfaction decreases over time. While the treatment effect on income satisfaction was 0.680 SD six months into the cash transfer program, it was nearly half as large (0.350 SD) at the end of the cash transfer program. (ii) While we find at most small treatment effects on work satisfaction during the first one and a half years (0.046 SD in wave 1, -0.012 SD in wave 2, and 0.12 SD in wave 3), cash transfers improve work satisfaction more substantially during the final one and a half years of the cash transfer program (0.24 SD in wave 4, 0.24 SD in wave 5, and 0.23 SD in wave 6).

Persistence after the cash transfer program Treatment effects remain present and sizable 6 and 18 months after the final cash transfers, see Figures 2 and 3, and Tables A.2 and A.3 in the Appendix. The MHW index improved by 0.379 SD during the program, by 0.331 SD six months after the program, and by 0.362 SD 18 months after the program, implying that, on average, 91% of the treatment effect on mental health and wellbeing persisted after monthly payments were discontinued.

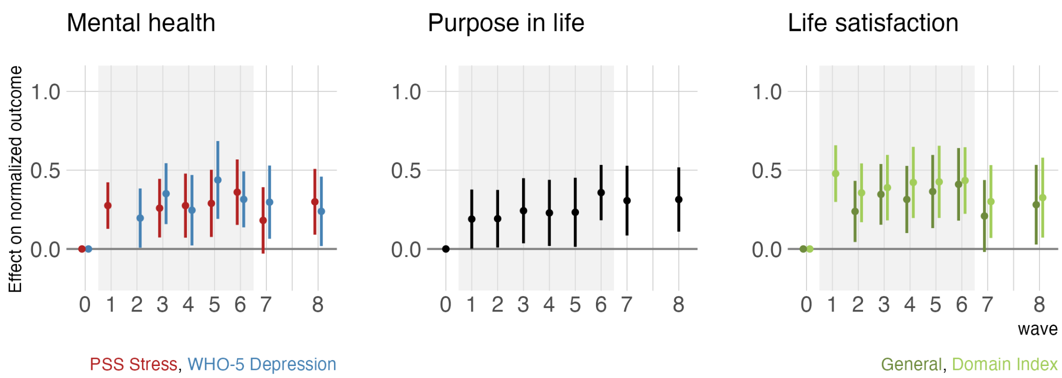

Figure 2: Dynamics of treatment effects on main mental health and wellbeing outcomes over the study period. Each panel displays estimated average treatment effects (ATE) in standard deviations, with 95% confidence intervals, for each survey wave. Outcomes are changes relative to baseline, so treatment effects are zero by construction at wave 0. Left panel: Mental health, decomposed into the WHO-5 Depression screener (blue) and perceived stress from the PSS (red). Center panel: Purpose in life (eudaimonic wellbeing). Right panel: Life satisfaction, decomposed into general life satisfaction (dark green) and the domain satisfaction index (light green). Treatment effects on mental health, purpose in life, and life satisfaction remain broadly stable over the three-year program, with no evidence of attenuation over time. Wave 7, conducted six months after the end of the program, and wave 8, conducted 18 months after the program, show persistent effects (see Tables B.3 and B.4). The WHO-5 questions and the general life satisfaction question were missing in wave $t = 1$, and the PSS questions were missing in wave $t = 2$.

The dynamics of the individual outcome components during and after the program are informative about the nature of this persistence. Not all domains of life satisfaction evolved in the same way. Income satisfaction — the outcome most directly tied to the cash transfer itself — shows a declining treatment effect already during the program and is no longer statistically significant 6 or 18 months after the program ends (0.140 SD and 0.130 SD, respectively). This trajectory is consistent with hedonic adaptation to the income gain, and with the mechanical loss of the income stream. By contrast, mental health, general life satisfaction, health satisfaction, sleep satisfaction, and purpose in life maintain treatment effects of 0.277–0.314 SD even 18 months after the transfers ceased, closely tracking their magnitudes during the program. Work satisfaction, which showed a delayed positive effect emerging only in the second half of the program, persists at 0.217 SD 18 months post-program.

The fading of income satisfaction alongside the stability of broader wellbeing and mental health improvements suggests that the lasting effects of the cash transfers are not simply a reflection of the additional income itself. Instead, the observed patterns suggest that the cash-transfers enabled treated participants to establish more beneficial sleeping habits and leisure activities, to improve their social relations, to build financial reserves and to improve their work situation during the study period, and that these changes may have become self-sustaining even

Figure 3: Dynamics of treatment effects on domain-specific satisfaction outcomes over the study period. Each panel displays estimated average treatment effects (ATE) in standard deviations, with 95% confidence intervals, for each survey wave. Outcomes are changes relative to baseline. Top row, left to right: health satisfaction, income satisfaction, leisure satisfaction. Bottom row: sleep satisfaction, social satisfaction, work satisfaction. Two notable dynamic patterns emerge. First, income satisfaction shows a declining treatment effect over time, falling from approximately 0.68 SD in wave 1 to 0.35 SD by wave 6, consistent with hedonic adaptation to the income gain. Second, work satisfaction exhibits a delayed positive effect: treatment effects are small during the first 18 months but increase substantially during waves 4–6 (approximately 0.23–0.24 SD). Health, sleep, and leisure satisfaction effects remain broadly stable throughout the program. Social satisfaction effects are directionally positive but not statistically significant at conventional levels in most waves.

after the income stream ended. In line with this interpretation, we present further explorative analyses on treatment effects on life changes and perceived agency in Section 4.

4 Agency and life changes

We present exploratory analyses of recipients' retrospective accounts of their experiences with the cash transfers, treatment effects on participants' self-reported household finance data, treatment effects on participants' time use data, and treatment effects on participants' perceived autonomy. Five patterns emerge that directly relate to drivers of mental health and wellbeing discussed in the literature. We hence view these patterns to provide suggestive evidence on the mechanisms behind the mental health and wellbeing improvements that we documented in the previous section – namely that cash transfers empower recipients to implement and experience meaningful life changes. The five patterns are:

(i) Improved financial security: According to recipients' own accounts of their experience with the cash transfers, and according to positive treatment effects on participants' saving behavior, the cash transfers improved the financial security of recipients. The corresponding literature identifies financial security to be a foundation for mental health and wellbeing (Jachimowicz et al., 2022; Guan et al., 2022).

(ii) Improved prosocial behavior: Recipient statements as well as positive treatment effects on participants' charitable giving and financial support of family and friends show that the cash transfers increased recipients' prosocial behavior. There is ample evidence that spending money on others improves wellbeing (Dunn et al., 2008; Hui et al., 2020).

(iii) Improved social life: Cash transfers increase the time participants spend with their friends. The related literature finds that a richer social life and less loneliness are cornerstones of good mental health and wellbeing (Harandi et al., 2017; Park et al., 2020; Waldinger and Schulz, 2023).

(iv) Improved recreation: Cash transfers tend to increase participants' sleeping time and their spending on leisure activities and traveling. Evidence from meta-analyses shows that better sleep (Scott et al., 2021) and new recreational activities and experiences (Kuykendall et al., 2015) improve mental health and wellbeing.

(v) Higher perceived autonomy: Recipient statements as well as treatment effects on perceived autonomy show that cash transfers improved participants' perceived agency and control over their lives. Correspondingly, ample evidence suggests that autonomy and freedom of choice are important prerequisites for wellbeing (Inglehart et al., 2008; Steckermeier, 2021).

In the following, we present the results behind patterns (i)-(v) more thoroughly.

Retrospective accounts of treated participants At the end of the survey in wave 6, that is, at the end of the cash transfer program, we asked cash-transfer recipients in a free form question to retrospectively reflect on how the cash transfers had impacted them. By making use of a free form question, we avoid anchoring participants' responses on specific aspects chosen by the researcher (Haaland et al., 2024). Instead, participants freely chose to write about what they deemed to be of relevance.

Three research assistants independently analysed participants' text responses and indicated for each response whether it mentions one or multiple of the following categories: Financial security, freedom/autonomy, wellbeing, cash transfers had no effect, changes in education, changes in work, changes in leisure activities, changes in health, changes in sleep, changes in social relationships, and changes in donations and financial support of others – i.e., reflecting the domain satisfactions.

We find that 72% of treated participants mentioned improvements in their financial security, and 67% mentioned that the cash transfers changed their lives. Regarding life changes, 37% treated participants mentioned their work and education, 38% leisure activities, and 63% donations and financial support of others. Relatedly, 25% of treated participants explicitly stated greater freedom of choice. Only 8% of treated participants stated that the cash transfers had no impact on their lives.

These findings provide suggestive support for patterns (i) through (v) by showing, that a substantial share of treated participants directly mentioned to experience/have experienced financial security and that a large share of treated participants mention specific life changes and some participants even mentioned increased freedom of choice abstractly.

Household finance All surveys included questions on monthly saving, biannual donations, biannual financial transfers given to family and friends in the past six months, current debt, and current assets, and the surveys of wave 3-6 also included questions on monthly spending on housing, energy costs, appliances, daily uses, mobility, leisure activities, apparel (clothes and shoes), and travel. The wording of these questions, and all other questions discussed below, is provided in Appendix On.5.

We report treatment effects on these outcomes, averaged across waves, in levels in Table 3 (and changes, when possible, in Table A.4 in Appendix A.3). We find positive and statistically significant treatment effects on participants' monthly saving, donations, transfers to family and friends, total assets, spending on apparel, travel and leisure activities. We do not find significant treatment differences on participants' debt, spending on housing, energy costs, mobility, appliances, and daily uses.

We first turn in more detail to the treatment effect on monthly saving. Participants in the control saved on average EUR 332 on a monthly basis during the cash-transfer program. Treated participants' monthly saving was EUR 779, that is EUR 447 greater than in the control. Accounting for baseline difference, the treatment effect amounts to EUR 455. Overall, treated participants saved more than one third of their monthly cash transfer during the cash transfer program. At the same time, we find no treatment effect on participants' debt, but the share of participants with assets less than EUR 10,000 at the end of the cash transfer program decreases significantly by 14pp due to the cash transfers (with Neyman and Fisher's exact p-value <0.01). Our results hence suggest that the cash transfers allowed recipients to improve their financial security, supporting pattern (i).

We next turn to the treatment effects on donations and financial support of family and friends. Control participants donated to charitable causes and financially supported their family and friends with, on average, EUR 62 per month. The cash transfers impacted prosocial giving of treated participants substantially during the program, increasing the sum of donations and financial support to family and friends by EUR 92.5 to overall EUR 154.5 per month. Accounting for baseline difference, the treatment effects amount to EUR 89 greater prosocial giving per month. Consequently, treated participants shared 7-8% of their monthly cash transfer with others. These results support pattern (ii).

Outcome	Treated	Control	ATE	SE	t-stat	p-N	p-F	n treated	n control
Financial household									
Donations (6mo), levels	170.310	73.105	97.205	27.822	3.494	0.000	0.018	107	1475
Transfers (6mo), levels	757.156	299.882	457.274	117.067	3.906	0.000	0.006	107	1474
Savings (monthly), levels	779.251	332.175	447.076	44.996	9.936	0.000	0.000	107	1473
Debt (stock), levels	14137.688	13703.791	433.897	3812.833	0.114	0.909	0.898	107	1474
Monthly spending									
Appliances, levels	421.294	496.737	-75.443	91.439	-0.825	0.409	0.410	107	1419
Daily needs, levels	340.421	327.582	12.838	15.498	0.828	0.407	0.414	107	1419
Apparel, levels	145.082	112.143	32.939	11.425	2.883	0.004	0.006	107	1419
Leisure, levels	184.168	147.135	37.033	13.971	2.651	0.008	0.010	107	1419
Mobility, levels	170.999	151.970	19.029	12.627	1.507	0.132	0.136	107	1419
Travel, levels	436.628	248.402	188.227	81.013	2.323	0.020	0.032	107	1419
Housing, levels	642.378	647.918	-5.540	32.441	-0.171	0.864	0.866	107	1419
Energy, levels	127.259	125.587	1.672	15.697	0.107	0.915	0.910	107	1419
Time use, h/w									
Chores, levels	7.241	7.305	-0.064	0.357	-0.179	0.858	0.868	107	1465
Education, levels	4.497	4.193	0.305	0.582	0.524	0.601	0.616	107	1465
Entertainment, levels	17.805	18.089	-0.284	0.958	-0.296	0.767	0.760	107	1465
Family, levels	6.806	6.056	0.750	0.718	1.045	0.296	0.314	107	1465
Friends, levels	9.834	8.512	1.322	0.538	2.457	0.014	0.016	107	1465
Partner, levels	13.491	11.766	1.724	1.449	1.190	0.234	0.274	107	1465
Sleep, levels	49.102	47.867	1.235	0.884	1.396	0.163	0.176	107	1465
Sport, levels	3.922	3.782	0.140	0.330	0.425	0.671	0.674	107	1465
Volunteering, levels	1.327	0.949	0.378	0.261	1.451	0.147	0.166	107	1465
Autonomy									
Autonomy, levels	4.024	3.706	0.320	0.065	3.824	0.000	0.000	107	1477

Table 3: Average treatment effects on household finance, monthly spending, time use, and perceived autonomy outcomes during the cash transfer program. Household finance outcomes are reported in EUR and averaged across waves during the program; donation and transfer variables refer to six-month periods, savings to monthly amounts, and debt to stocks. Monthly spending variables (available in waves 3–6) are reported in EUR per month. Time use outcomes (available in waves 0–3 and 5) are reported in hours per week. Perceived autonomy is measured on a 1–11 Likert scale and reported in standard deviations. All outcomes are in levels (without baseline adjustment). Inference is based on heteroskedasticity-robust standard errors (SE) and both Neyman (p-N) and Fisher's exact (p-F) p-values. Treated participants saved approximately one-third of their monthly cash transfer, shared 7–8% with others through donations and financial support, spent significantly more on apparel, leisure, and travel, spent 1.3 more hours per week with friends, and reported significantly higher perceived autonomy (0.32 SD).

To prevent excessive survey burden, we did not cover participants' consumption rigorously. Based on questions intended to provide a broad overview of consumption, however, we find that treated participants spent on average EUR 33 per month more on clothes and shoes, EUR 37 per month more on leisure activities, and EUR 188 per month more on travelling. The latter two treatment effects amount to EUR 225 or roughly 19% of the monthly cash transfers, suggesting that the cash transfers allowed treated participants to have a more active leisure time, supporting pattern (iv).

Time use In waves 0-3 and 5, we surveyed participants' time use (in hours per week) on chores, education, entertainment, family, friends, partner, sleep, sport, and volunteering. We report treatment effects on these outcomes, averaged across waves, in levels in Table 3 (and changes in Table A.4 in Appendix A.3).

We find positive treatment effects on time spent with friends. The control group spent, on average, roughly 8.6 hours with friends per week. Treated participants spent 1.3 hours more time with friends per week than the control, an increase of 15%. Accounting for baseline differences, the treatment effect increases to slightly more than 2 hours. These results support pattern (iii) stated above.

We also find a positive treatment effect on participants' sleeping time when considering changes relative to the baseline: According to this estimated treatment effect, treated participants slept 2.3 hours more per week relative to control participants. On a nightly basis, this amounts to roughly 20 minutes longer sleep. Consistent with this finding, treated participants report greater sleep satisfaction, an important mediator for the positive effects on longer sleep on greater mental health and wellbeing (Scott et al., 2021). While these treatment effects support pattern (iv) reported above, we do not find a statistically significant treatment effect on sleeping time in levels. According to this insignificant estimate, treated participants slept 1.2 hours longer per week than control participants.

Perceived autonomy Starting in wave $t = 1$, we elicited participants perceived autonomy in each wave. We asked participants how autonomous they feel in their own life. We report the treatment effect on this outcome in levels in the final row of Tables 3. We find that cash transfers improve perceived autonomy by 0.320 SD on average across waves. Consistent with participants' retrospective accounts on life changes and greater freedom of choice, and consistent with the positive treatment effects on participants' spending and time use, this finding suggests that participants experienced greater agency, allowing them to shape their lives to a greater extent according to their values and needs. This treatment effect hence supports pattern (v) documented above.

5 Related literature

Our results contribute to several literatures. In particular, our results differ from contemporaneous RCTs on cash transfers in the US and Canada, which do not find that cash transfers improve mental health. We argue that the difference in results may be caused by the fact that cash transfers, in our case, allow recipients to experience greater agency and implement meaningful life changes. In addition, our paper contributes to the broader literature on the effects of money on mental health and wellbeing. We argue that cash transfers that are unconditional, regular, and guaranteed, are particularly suited to have positive effects on recipients. We close our discussion with more specific contributions on the link between autonomy and wellbeing and on the stability of wellbeing improvements over time.

The effect of cash transfers on mental health and wellbeing The literature on cash transfers in low and mid-income countries finds that cash transfers allow recipients to improve mental health and wellbeing (Haushofer and Shapiro, 2016; Christian et al., 2019; Romero et al., 2021). A recent meta study reports modest improvements in mental health of 0.07 SD and in life satisfaction of 0.13 SD (McGuire et al., 2022). Prominent authors have argued that there is a smaller scope of mental health and wellbeing improvements in richer countries (Ryan and Deci, 2001; Diener et al., 2009; Easterlin and Sawangfa, 2010; Kahneman and Deaton, 2010), where basic needs tend to be satisfied, comparatively generous social benefit programs are in place, and adaption and satiation effects limit potential improvements, as material goals rise with income gains (Easterlin and Sawangfa, 2010; Kahneman and Deaton, 2010). Indeed the contemporaneous literature on cash transfers in high-income countries finds no evidence for durable improvements in mental health (West and Castro, 2023; Dwyer et al., 2023; Jaroszewicz et al., 2024; Miller et al., 2024; Balakrishnan et al., 2024).

We contribute to this literature by showing that cash transfers can lead to large and durable improvements in mental health and wellbeing. Our findings suggest that cash transfers need to be effectively generous at the individual level and preserve recipients' discretionary control over the transfer. The most comparable study is a contemporaneous cash-transfer RCT in the US that paid USD 1,000 on a monthly basis for three years (Miller et al., 2024; Vivalt et al., 2024; Bartik et al., 2024). The single monthly transfer of USD 1,000 accrued to households with, on average, three members, making it likely that the effective transfer available to any individual recipient was well below the nominal amount. In our setting, by contrast, each participant received EUR 1,200 individually, as we restricted the study to single-person households.

We argue that individual-level transfers ensure that each recipient retains full discretionary control over the transfer, without having to negotiate its use within a shared household budget. This individual agency is likely a precondition for the kind of personally meaningful life changes that our evidence suggests drive lasting improvements in mental health and wellbeing. Several patterns in the comparison with Miller et al. (2024); Vivalt et al. (2024); Bartik et al. (2024) support this interpretation. First, cash transfers in the US setting do not lead to the type of life changes that we observe for our sample. Bartik et al. (2024) document that recipients' spending patterns are consistent with the transfer being shared within the household rather than being used to reshape individual life domains. By contrast, our recipients — who retained full individual control — invested in social relationships, recreational activities, and personal financial security. Evidence from a different context reinforces this point: Baird et al. (2025) find that cash transfers to adolescent girls in Uganda failed to improve mental health when recipients had to divert funds towards essential family needs rather than pursuing their own goals. Second, cash transfer recipients in the US setting reduce labor force participation (Vivalt et al., 2024), whereas we find no such reduction in our companion paper (Bernhard et al., 2025). This difference likely matters for mental health and wellbeing. Employment provides time structure, social contact, collective purpose, status, and activity that benefit mental health independently of income (Jahoda, 1981, 1982; Paul et al., 2023). A reduction in labor force participation may therefore offset potential wellbeing gains from the transfer, while the absence of such a reduction in our setting allows the positive effects of greater financial security and agency to operate without this countervailing force.

Beyond the individual versus household distinction, the two settings differ along several baseline dimensions. First, the US sample was more economically disadvantaged: only 58% of participants were employed at baseline, and household net worth was close to zero or negative when excluding real estate (Bartik et al., 2024). Moreover, the study included a substantial share of single parents — 22% of the sample were single mothers (Vivalt et al., 2024) — who face particularly severe financial constraints and competing demands on any additional income. By contrast, 94% of our sample were working for pay and the average net wealth of our sample was positive. Second, institutional differences likely put the US sample at another disadvantage relative to our study sample: Germany's largely universal health insurance, comprehensive social safety net, and stronger worker protections mean that many of the basic needs that cash transfers might otherwise absorb — medical expenses, income volatility, housing insecurity — are already partially addressed through existing institutions.

Overall, this suggests that what matters for lasting mental health and wellbeing improvements is not the severity of baseline deprivation per se, but whether the cash transfer is effectively generous enough to empower individual agency and meaningful life changes. The German institutional environment may indeed facilitate this mechanism because basic needs are largely covered and additional income can be directed toward discretionary life improvements. In this sense, the more comprehensive safety net does not reduce the scope for cash transfers to make a difference. It channels their impact toward the domains — social relationships, recreation, autonomy — that our evidence suggests matter for sustained mental health and wellbeing improvements.

Lottery winnings versus cash transfers We also contribute to the literature on whether money more generally improves wellbeing in high-income countries. Much of this literature relies on lottery winnings to estimate the causal effects (Gardner and Oswald, 2007; Kuhn et al., 2011; Apouey and Clark, 2015; Raschke, 2019; Lindqvist et al., 2020; Dwyer and Dunn, 2022). The reported findings are mixed: mental health is found to be affected positively (Gardner and Oswald, 2007; Apouey and Clark, 2015), not at all (Lindqvist et al., 2020) or, potentially even, negatively (Raschke, 2019); life satisfaction is reported to be affected positively (Apouey and Clark, 2015; Lindqvist et al., 2020; Dwyer and Dunn, 2022), or not at all (Kuhn et al., 2011). For instance, large lottery winnings of USD 100,000 or more in Sweden do not improve mental health and improve long-term life satisfaction by merely 0.034 SD for every USD 100,000 of winning (Lindqvist et al., 2020). Lottery winnings, even when large, represent one-off wealth shocks that recipients must actively manage and allocate. As one-off events, they are particularly susceptible to hedonic adaptation (Brickman et al., 1978; Frederick and Loewenstein, 1999), and the burden of managing a large windfall can itself be a source of psychological distress (Raschke, 2019).

Our results suggest that regular, unconditional, guaranteed, and individual cash transfers may be particularly well-suited to improve wellbeing. Guaranteed and regular cash transfers that continue into the future enhance the predictability of financial security, help to sustain reasonable spending habits, and limit potential stress and anxiety related to the management of large lump sum payments. They appear to limit adaptation and satiation effects, potentially by providing continuous reminders and positive reinforcement of financial security. In addition, when such transfers accrue to individuals rather than to households, recipients retain full discretionary control, allowing them to direct resources toward personally meaningful life changes rather than negotiating their use within a shared household budget.

Autonomy and wellbeing improvements Regular, unconditional, guaranteed, and individual cash transfers ease financial constraints. As a result, recipients may find themselves with more freedom to shape their lives according to their tastes, values, and psychological needs (Maslow, 1943). In turn, they may attribute this increased sense of control to their own autonomous disposition, rather than to changes in circumstances (Kraus et al., 2009). While previous work shows that the correlation between perceived autonomy and life satisfaction is often greater in richer than in poorer countries (Inglehart et al., 2008), we add more direct evidence on the link between autonomy and wellbeing improvements.

Stability of wellbeing improvements Our findings imply relatively stable wellbeing improvements. Previous research, mostly based on correlational evidence, suggests otherwise: Models of adaptation effects posit that, as people get used to greater income, wellbeing improvements attenuate (Vendrik, 2013). Models of satiation effects, relatedly, suggest that wellbeing improvements due to income gains are less pronounced, the more income is already available (Easterlin and Sawangfa, 2010; Kahneman and Deaton, 2010). While we do find some evidence for adaptation effects on income satisfaction, we find no evidence for adaptation effects in mental health, purpose in life, and in the other domains of life satisfaction, consistent with more recent correlational evidence against adaptation effects (Kaiser, 2020; Stevenson and Wolfers, 2013). This suggests that wellbeing improvements caused by financial improvements may be more stable than *previously* thought – in particular if they lead to lasting adjustments in lifestyle choices.

6 Discussion

In this section, we discuss the internal and external validity of our findings. We show that our findings are robust to multiple-hypothesis-testing adjustments and are unlikely the results of alternative explanations. We discuss limitations of the generalizability of our findings.

Multiple hypothesis testing Following our preregistration and the methods discussed in Section 2.3, we apply the BH procedure to control for multiple hypothesis testing within the group of mental health and wellbeing outcomes. Our main results are based on the $m = 56$

estimated treatment effects stated in Table 2, Table A.1, Table A.2, and Table A.3. 42 of the 56 estimated treatment effects in these tables are significantly different from zero. We continue to determine 41 of these 42 treatment effects to be statistically significant when applying BH critical values. Only the treatment effect on sleep satisfaction six months after the cash transfer program is no longer statistically significant, its p-value is not below its BH critical value.

Experimenter demand, social desirability, and Hawthorne effects The internal validity of experiments with human participants may, in principle, suffer from experimenter demand, social desirability and Hawthorne effects (Levitt and List, 2011; De Quidt et al., 2018; Bursztyn et al., 2025). Since our implementation partner, who financed the cash transfers, actively supports basic income policies, a potential experimenter demand or social desirability effect would be that treated participants might self-report greater mental health and wellbeing to lend support to basic income policies more generally. If our findings are due to Hawthorne effects, participants change their behavior only because they are experimentally observed. The following observations suggest that experimenter demand, social desirability and Hawthorne effects do not account for our findings.

First, our experimental design limits the potential roles of experimenter demand, social desirability and Hawthorne effects. To limit demand and social desirability effects, we explicitly asked participants to respond accurately to factual questions, stated that there are no right or wrong answers to subjective questions, and used a third-party survey company to implement our surveys. Regarding Hawthorne effects, treated and control participants are observed to the same extent. This makes explanations of our findings based on Hawthorne effects, where outcomes are moved by observation rather than treatment, implausible.

Second, if experimenter demand and social desirability effects are relevant in the context of our RCT, they should similarly affect participants' self-reported labor supply. This follows from the public debate about cash transfers in Germany at the start of our RCT, which focused primarily on whether the cash transfers will severely reduce recipients' labor supply, that is, make recipients "lazy." We analyse the effect of the cash transfers on participants' labor supply in a companion paper (Bernhard et al., 2025). Importantly, our analyses on labor supply rely not only on self reports, but also on administrative records (based on the Integrated Labor Market Biographies dataset constructed by the German Institute for Employment Research, see Schmucker and Vom Berge, 2025).[6] We find no effects on labor force participation and small negative effects on working hours. Importantly, these effects are consistent between self reports and administrative records (Bernhard et al., 2025). That we find no inflated self reports on labor supply relative to participants' administrative records suggests that experimenter demand and social desirability concerns play a limited role in the context of our RCT.

Third, we find no treatment effect on participants' political support for basic income policies.

[6] We asked participants after the first year of the cash transfer program whether they allow us to link their treatment status with their administrative records. Participants did not know we would ask this during the first year. The administrative records, however, allow us to observe labor supply of participants also retrospectively during the first year of the cash transfer program.

At the end of the final wave, we asked participants the following question: "Do you support the idea of a universal basic income for all citizens?"[7] Participants could respond on a 1-4 scale, with 1 = "yes, absolutely," 2 = "rather yes," 3 = "rather no," 4 = "no, absolutely not." The average response of treated participants is 1.928 and of control participants is 1.964. The treatment difference is -0.035, which is quite small and not statistically significantly different from zero, with a one-sided Neyman p-value of 0.33 and one-sided Fisher's exact p-value of 0.3.[8] This finding contradicts explanations based on experimenter demand and social desirability, where recipients try to give answers that lend support to basic income policies.

Fourth, the dynamics over time of treatment effects on wellbeing are not consistent with experimenter demand, social desirability and Hawthorne effects, either. These effects cannot explain the decreasing treatment effect on income satisfaction over time, nor the increasing treatment effects on work satisfaction.

Finally, experimenter demand, social desirability, and Hawthorne effects are also not consistent with the overall results of cash-transfer RCTs in high-income countries. If experimenter demand, social desirability, and Hawthorne effects would provide reasonable explanations for our results, these effects should produce similar results in comparable cash-transfer RCT. Miller et al. (2024), however, do not find positive treatment effects on mental health in the US, arguably because the cash transfers in their setting did not empower agency and life changes, see our discussion in Section 5 above. Importantly, the US RCT was also publicly linked to the debate on basic income policies.[9] Experimenter demand and social desirability effects should hence equally be of concern in the US setting. That the results of the US RCT differ from our results therefore suggests that experimenter demand, social desirability, and Hawthorne effects may be less relevant in these long-term RCT settings.

Based on these analyses and observations, we conclude that our results are not likely to be driven by experimenter demand, social desirability, and Hawthorne effects.

Attrition We encountered no attrition during waves 1 to 6 in our treatment group and some attrition in our control group, where survey response rates declined slightly. Nonetheless, 71% of our control group completed all 6 surveys, 79% completed at least 5 out of 6 waves, 88% at least 3 out of 6 waves, and 97% at least 1 out of 6 waves. Based on the following two analyses, we however conclude that attrition does not appear to be selective in a way that would impact our findings.

If attrition were selective, this might cause bias in our treatment effect estimates. We might find significant effects, even if in truth (without selection) there is no effect. As a placebo

[7] In German, we asked: "Befürworten Sie die Idee eines bedingungslosen Grundeinkommens für alle Bürger/innen?"

[8] Since demand effects predict greater support for treated participants, we conducted one-sided tests, in favor of finding significant demand and social desirability effects.

[9] The US RCT was financed by Y Combinator. Sam Altman, president of Y Combinator at the time the RCT was conceived, publicly stated that the RCT should speak to the debate on basic income and argued that he is "fairly confident that at some point in the future, [...] we're going to see some version of [basic income] at a national scale" (Altman, 2016).

test for this possibility, we estimate treatment effects on our baseline covariates (where true treatment effects are by construction equal to 0), while artificially restricting our sample to only those individuals without missing data for any subsequent wave. The P-values corresponding to these estimates are shown in Figure A.1 in Appendix A.4. We find no significant effects for the restricted sample. This increases our confidence that our findings are not driven by selective attrition.

As a second test for selective attrition, we restrict attention to the control group. Within this group, we compare the average of our main outcome variables (mental health, purpose in life, life satisfaction) at baseline (wave 0), across observations with different numbers of missing waves. If attrition were selective, we would expect that these means vary across the number of waves missing, see Table A.5 in Appendix A.4. We find no differences across number of waves missing.

Based on these two analyses, we conclude that our results are not likely to be affected by selective attrition.

Disappointed control An alternative explanation for our findings is that the control group's mental health and wellbeing declined due to disappointment at not receiving the cash transfers, rather than the treatment group truly improving. Based on several observations, we find this "disappointed control" account unlikely, and instead attribute the results to genuine mental health and wellbeing improvements of the treatment group.

First, the pattern of mental health and wellbeing improvements does not match what one would expect if control-group disappointment were driving the effects. Many of the observed mental health and wellbeing improvements are due to positive improvements in the treatment group rather than declines in the control group, see Table 2. For instance, the treatment group's mental health increased by about 0.322 SD from baseline, whereas the control group's remained essentially unchanged (+0.02 SD), yielding a sizable treatment effect (0.305 SD). Similar patterns hold for outcomes like income, leisure, and social satisfaction. In contrast, for outcomes such as general life satisfaction, sleep satisfaction, and purpose in life, the control group reported small declines over time while the treated group improved, contributing jointly to the treatment effects. Only work and health satisfaction declined over time in both treatment and control groups, with more pronounced declines for the control. This selective pattern of improvement is not consistent with a uniform "disappointed control" effect that would broadly depress the control group's mental health wellbeing.

Second, the analyses stated in the previous paragraph cannot rule out an extended version of control-group disappointment that combines control-group disappointment with population-wide shocks to mental health and wellbeing that differ between the different outcomes. Assume that the entire German population experienced shocks (unrelated to the cash transfers) that caused large improvements of mental health, income satisfaction, and leisure satisfaction, medium-sized improvements of purpose in life and general life satisfaction improvements, and no improvements of work and health satisfaction during the program. In combination with control-group disappointment, such shocks could in principle account for our results. To address also this extended version of control-group disappointment, we turn to the SOEP and find *no* evidence for such shocks in the German population or in the study population (the part of the German population that would have been eligible in 2021 to participate in our study), see Appendix A.6. We hence conclude that also the extended version of control-group disappointment is not consistent with our findings.

Finally, our analyses show that recipients used their cash transfers in ways that meaningfully changed their lives, see Section 4. These tangible changes align with improved mental health and wellbeing of treated participants. Importantly, changes in each one of the domains that we discussed—improved financial security, prosocial giving, social connectedness, better sleep, and empowered agency—are independently linked in the literature to better mental health and wellbeing outcomes. Our results are hence not only inconsistent with control disappointment, but our explorative analyses provide evidence for why the cash transfers allowed the treatment group to improve their mental health and wellbeing, namely empowered agency and life changes.

Based on these analyses and observations, we conclude that our results are not likely to be driven by a disappointed control.

External validity The cash transfer program covered in our RCT is resource intensive, costing in total EUR 4.6 million. The size of the treatment group is hence limited, so that we cannot study general equilibrium effects. Our results are furthermore based on a non-representative sample, see Table A.6 in Appendix A.5, which potentially limits generalizability. Our effect sizes may also not generalize to settings where cash transfers are either less or more generous (both in terms of the monthly amounts and the time they are paid out). Cash transfers might in practice be combined with other policy changes, such as increases in taxation to finance the cash transfers, that are not covered in our RCT.

A further limitation concerns the duration of the cash transfer program. Our participants received monthly transfers for three years, a period that seems to have been long enough to enable meaningful life changes that lead to durable improvements in mental health and wellbeing. It is unclear whether shorter programs would generate the same type of agency-driven improvements, and whether longer programs would generate different behavioral responses, including stronger labor supply effects (than we report in Bernhard et al., 2025) or different patterns of hedonic adaptation.

By restricting our sample to single-person households, we ensured that each recipient had full individual control over the transfer. This makes our treatment more representative of genuinely individual cash transfer programs — the form most commonly discussed in policy proposals (Pateman, 2004; Van Parijs and Vanderborght, 2017) — than studies where a single transfer is shared among multiple household members. To the extent that the policy debate centers on individual-level transfers, our design provides a more informative test of their potential effects on mental health and wellbeing, even if our sample does not generalize straightforwardly to multi-person households or the broader population.

7 Conclusion

We show that unconditional, regular, guaranteed, and individual cash transfers improve recipients' mental health, purpose in life, and life satisfaction. Exploratory analyses suggest that these improvements are mediated by greater personal agency and subsequent life changes. Our findings contribute to policy debates on how high-income countries can directly improve their citizens' wellbeing.

First, they suggest that such transfers can enhance individuals' agency and better equip them to navigate the profound transformations of the 21st century. Optimal tax and transfer theory in economics (Saez, 2002; Chetty, 2009; Kleven, 2021) also incorporates the costs of transfer programs to assess its overall appeal. As cash transfers can be financed by raising progressive income taxes, or by some other form of taxation, the costs of cash transfer programs depend in particular on the extent to which the tax base is affected via labor supply responses to the policy. We explore these questions of labor market effects and optimal policy design in greater detail in Bernhard et al. (2025).

Second, our finding that treatment effects persist at roughly 91% of their during-program magnitude 18 months after the final transfer suggests that cash transfer programs need *not* be permanent to generate lasting benefits. A time-limited program of sufficient duration may allow recipients to implement life changes that sustain improvements in mental health and wellbeing beyond the program itself, substantially reducing the fiscal cost relative to a permanent transfer.

References

Altman, Sam, "Basic Income," https://www.ycombinator.com/blog/basic-income 27/01 2016.

Annan, Francis and Belinda Archibong, "The value of communication for mental health," Technical Report, National Bureau of Economic Research 2023.

Apouey, Benedicte and Andrew E Clark, "Winning big but feeling no better? The effect of lottery prizes on physical and mental health," *Health economics*, 2015, *24* (5), 516–538.

Athey, Susan and Guido W Imbens, "The econometrics of randomized experiments," in "Handbook of economic field experiments," Vol. 1, Elsevier, 2017, pp. 73–140.

Baird, Sarah, Berk Özler, Chiara Dell'Aira, Luca Parisotto, and Danish Us-Salam, "Therapy, mental health, and human capital accumulation among adolescent girls in Uganda," *Journal of Development Economics*, 2025, *176*, 103473.

_ , Jacobus De Hoop, and Berk Özler, "Income shocks and adolescent mental health," *Journal of Human Resources*, 2013, *48* (2), 370–403.

Balakrishnan, Sidhya, Sewin Chan, Sara Constantino, Johannes Haushofer, and Jonathan Morduch, "Household Responses to Guaranteed Income: Experimental Evidence from Compton, California," Technical Report, National Bureau of Economic Research 2024.

Bartik, Alexander W, Elizabeth Rhodes, David E Broockman, Patrick K Krause, Sarah Miller, and Eva Vivalt, "The Impact of Unconditional Cash Transfers on Consumption and Household Balance Sheets: Experimental Evidence from Two US States," Technical Report, National Bureau of Economic Research 2024.

Bartling, Björn, Ernst Fehr, and Holger Herz, "The intrinsic value of decision rights," *Econometrica*, 2014, *82* (6), 2005–2039.

Benjamini, Yoav and Yosef Hochberg, "Controlling the false discovery rate: a practical and powerful approach to multiple testing," *Journal of the Royal statistical society: series B (Methodological)*, 1995, *57* (1), 289–300.

Bernhard, Sarah, Sandra Bohmann, Susann Fiedler, Maximilian Kasy, Jürgen Schupp, and Frederik Schwerter, "Basic Income and Labor Supply: Evidence from an RCT in Germany," Technical Report, DIW Berlin, German Institute for Economic Research 2025.

Bidadanure, Juliana Uhuru, "The political theory of universal basic income," *Annual Review of Political Science*, 2019, *22* (1), 481–501.

Brickman, Philip, Dan Coates, and Ronnie Janoff-Bulman, "Lottery winners and accident victims: Is happiness relative?," *Journal of Personality and Social Psychology*, 1978, *36* (8), 917–927.

Bursztyn, Leonardo, Ingar K Haaland, Nicolas Röver, and Christopher Roth, "The Social Desirability Atlas," Technical Report, National Bureau of Economic Research 2025.

Chetty, Raj, "Sufficient Statistics for Welfare Analysis: A Bridge Between Structural and Reduced-Form Methods," *Annual Review of Economics*, 2009, *1* (1), 451–488.

Christian, Cornelius, Lukas Hensel, and Christopher Roth, "Income shocks and suicides: Causal evidence from Indonesia," *Review of Economics and Statistics*, 2019, *101* (5), 905–920.

Cohen, S, "Perceived stress in a probability sample of the United States," *The social psychology of health*, 1988, pp. 31–67.

Cohen, Sheldon, Tom Kamarck, and Robin Mermelstein, "A global measure of perceived stress," *Journal of health and social behavior*, 1983, pp. 385–396.

De Quidt, Jonathan, Johannes Haushofer, and Christopher Roth, "Measuring and bounding experimenter demand," *American Economic Review*, 2018, *108* (11), 3266–3302.

Diener, E, R Lucas, U Schimmack, and J Helliwell, *Well-being for public policy*, Oxford University Press, 2009.

Diener, Ed and Robert Biswas-Diener, "Will money increase subjective well-being?," *Social indicators research*, 2002, *57*, 119–169.

Dunn, Elizabeth W, Lara B Aknin, and Michael I Norton, "Spending money on others promotes happiness," *Science*, 2008, *319* (5870), 1687–1688.

Dwyer, Ryan, Anita Palepu, Claire Williams, Daniel Daly-Grafstein, and Jiaying Zhao, "Unconditional cash transfers reduce homelessness," *Proceedings of the National Academy of Sciences*, 2023, *120* (36), e2222103120.

Dwyer, Ryan J and Elizabeth W Dunn, "Wealth redistribution promotes happiness," *Proceedings of the National Academy of Sciences*, 2022, *119* (46), e2211123119.

Easterlin, Richard A and Onnicha Sawangfa, "Happiness and economic growth: Does the cross section predict time trends? Evidence from developing countries," *International differences in well-being*, 2010, pp. 166–216.

Frederick, Shane and George Loewenstein, "Hedonic adaptation," in Daniel Kahneman, Ed Diener, and Norbert Schwarz, eds., *Well-Being: The Foundations of Hedonic Psychology*, New York: Russell Sage Foundation, 1999, pp. 302–329.

Frey, Bruno S, Matthias Benz, and Alois Stutzer, "Introducing procedural utility: Not only what, but also how matters," *Journal of Institutional and Theoretical Economics (JITE)/Zeitschrift für die gesamte Staatswissenschaft*, 2004, pp. 377–401.

Fryers, Tom, David Melzer, and Rachel Jenkins, "Social inequalities and the common mental disorders: a systematic review of the evidence," *Social psychiatry and psychiatric epidemiology*, 2003, *38*, 229–237.

Gardner, Jonathan and Andrew J Oswald, "Money and mental wellbeing: A longitudinal study of medium-sized lottery wins," *Journal of health economics*, 2007, *26* (1), 49–60.

Guan, Naijie, Alessandra Guariglia, Patrick Moore, Fangzhou Xu, and Hareth Al-Janabi, "Financial stress and depression in adults: A systematic review," *PloS one*, 2022, *17* (2), e0264041.

Haaland, Ingar K, Christopher Roth, Stefanie Stantcheva, and Johannes Wohlfart, "Measuring what is top of mind," Technical Report, National Bureau of Economic Research 2024.

Harandi, Tayebeh Fasihi, Maryam Mohammad Taghinasab, and Tayebeh Dehghan Nayeri, "The correlation of social support with mental health: A meta-analysis," *Electronic physician*, 2017, *9* (9), 5212.

Haushofer, Johannes and Jeremy Shapiro, "The short-term impact of unconditional cash transfers to the poor: experimental evidence from Kenya," *The Quarterly Journal of Economics*, 2016, *131* (4), 1973–2042.

Holt-Lunstad, Julianne, "Social connection as a critical factor for mental and physical health: evidence, trends, challenges, and future implications," *World Psychiatry*, 2024, *23* (3), 312–332.

Hui, Bryant PH, Jacky CK Ng, Erica Berzaghi, Lauren A Cunningham-Amos, and Aleksandr Kogan, "Rewards of kindness? A meta-analysis of the link between prosociality and well-being.," *Psychological Bulletin*, 2020, *146* (12), 1084.

Inglehart, Ronald, Roberto Foa, Christopher Peterson, and Christian Welzel, "Development, freedom, and rising happiness: A global perspective (1981–2007)," *Perspectives on psychological science*, 2008, *3* (4), 264–285.

Jachimowicz, Jon M, Erin L Frey, Sandra C Matz, Bertus F Jeronimus, and Adam D Galinsky, "The sharp spikes of poverty: Financial scarcity is related to higher levels of distress intensity in daily life," *Social Psychological and Personality Science*, 2022, *13* (8), 1187–1198.

Jahoda, Marie, "Work, employment, and unemployment: Values, theories, and approaches in social research.," *American psychologist*, 1981, *36* (2), 184.

_ , "Employment and unemployment," *Cambridge books*, 1982.

Jaroszewicz, Ania, Jon Jachimowicz, Oliver Hauser, and Julian Jamison, "How effective is (more) money? Randomizing unconditional cash transfer amounts in the US," *Working Paper*, 2024.

Kahneman, Daniel and Angus Deaton, "High income improves evaluation of life but not emotional well-being," *Proceedings of the national academy of sciences*, 2010, *107* (38), 16489–16493.

Kaiser, Caspar, "People do not adapt. New analyses of the dynamic effects of own and reference income on life satisfaction," *Journal of Economic Behavior & Organization*, 2020, *177*, 494–513.

Kleven, Henrik J, "Sufficient statistics revisited," *Annual Review of Economics*, 2021, *13* (1), 515–538.

Kraus, Michael W, Paul K Piff, and Dacher Keltner, "Social class, sense of control, and social explanation.," *Journal of personality and social psychology*, 2009, *97* (6), 992.

Kuhn, Peter, Peter Kooreman, Adriaan Soetevent, and Arie Kapteyn, "The effects of lottery prizes on winners and their neighbors: Evidence from the Dutch postcode lottery," *American Economic Review*, 2011, *101* (5), 2226–2247.

Kuykendall, Lauren, Louis Tay, and Vincent Ng, "Leisure engagement and subjective well-being: A meta-analysis.," *Psychological bulletin*, 2015, *141* (2), 364.

Levitt, Steven D and John A List, "Was there really a Hawthorne effect at the Hawthorne plant? An analysis of the original illumination experiments," *American Economic Journal: Applied Economics*, 2011, *3* (1), 224–238.

Lindqvist, Erik, Robert Östling, and David Cesarini, "Long-run effects of lottery wealth on psychological well-being," *The Review of Economic Studies*, 2020, *87* (6), 2703–2726.

Ma, Ruimin, Farhana Mann, Jingyi Wang, Brynmor Lloyd-Evans, Joanna Terhune, Ahmad Al-Shihabi, and Sonia Johnson, "Loneliness and the onset of new mental health problems in the general population," *Social Psychiatry and Psychiatric Epidemiology*, 2022, *57* (12), 2161–2178.

Marinescu, Ioana, "No strings attached: The behavioral effects of US unconditional cash transfer programs," 2018.

Marmot, Michael, "Fair society, healthy lives," *Fair society, healthy lives*, 2013, pp. 1–74.

Maslow, AH, "A theory of human motivation," *Psychological Review google schola*, 1943, *2*, 21–28.

McGuire, Joel, Caspar Kaiser, and Anders M Bach-Mortensen, "A systematic review and meta-analysis of the impact of cash transfers on subjective well-being and mental health in low-and middle-income countries," *Nature Human Behaviour*, 2022, *6* (3), 359–370.

Miller, Sarah, Elizabeth Rhodes, Alexander W Bartik, David E Broockman, Patrick K Krause, and Eva Vivalt, "Does Income Affect Health? Evidence from a Randomized Controlled Trial of a Guaranteed Income," Technical Report, National Bureau of Economic Research 2024.

Moore, Ryan T and Keith Schnakenberg, "blockTools: Blocking, assignment, and diagnosing interference in randomized experiments. R package version 0.6-3," 2021.

Park, Caroline, Amna Majeed, Hartej Gill, Jocelyn Tamura, Roger C Ho, Rodrigo B Mansur, Flora Nasri, Yena Lee, Joshua D Rosenblat, Elizabeth Wong et al., "The effect of loneliness on distinct health outcomes: a comprehensive review and meta-analysis," *Psychiatry research*, 2020, *294*, 113514.

Pateman, Carole, "Democratizing citizenship: some advantages of a basic income," *Politics & society*, 2004, *32* (1), 89–105.

Paul, Karsten Ingmar, Hannah Scholl, Klaus Moser, Andrea Zechmann, and Bernad Batinic, "Employment status, psychological needs, and mental health: Meta-analytic findings concerning the latent deprivation model," *Frontiers in psychology*, 2023, *14*, 1017358.

Prince, Martin, Vikram Patel, Shekhar Saxena, Mario Maj, Joanna Maselko, Michael R Phillips, and Atif Rahman, "No health without mental health," *The lancet*, 2007, *370* (9590), 859–877.

Raschke, Christian, "Unexpected windfalls, education, and mental health: Evidence from lottery winners in Germany," *Applied Economics*, 2019, *51* (2), 207–218.

Richardson, Safiya, Jonathan A Shaffer, Louise Falzon, David Krupka, Karina W Davidson, and Donald Edmondson, "Meta-analysis of perceived stress and its association with incident coronary heart disease," *The American journal of cardiology*, 2012, *110* (12), 1711–1716.

Richter, David, Julia Rohrer, Maria Metzing, Wiebke Nestler, Michael Weinhardt, and Jürgen Schupp, "SOEP scales manual (updated for SOEP-Core v32. 1)," Technical Report, SOEP survey papers 2017.

Romero, Jimena, Kristina Esopo, Joel McGuire, and Johannes Haushofer, "The effect of economic transfers on psychological well-being and mental health," *Mimeo*, 2021.

Ryan, Richard M and Edward L Deci, "On happiness and human potentials: A review of research on hedonic and eudaimonic well-being," *Annual review of psychology*, 2001, *52* (1), 141–166.

_ and _ , *Self-determination theory: Basic psychological needs in motivation, development, and wellness*, Guilford publications, 2017.

Saez, Emmanuel, "Optimal income transfer programs: intensive versus extensive labor supply responses," *The Quarterly Journal of Economics*, 2002, *117* (3), 1039–1073.

Schmucker, Alexandra and Philipp Vom Berge, "Stichprobe der integrierten arbeitsmarktbiografien (siab) 1975-2023," Technical Report, Institut für Arbeitsmarkt-und Berufsforschung (IAB), Nürnberg [Institute for . . . 2025.

Scott, Alexander J, Thomas L Webb, Marrissa Martyn-St James, Georgina Rowse, and Scott Weich, "Improving sleep quality leads to better mental health: A meta-analysis of randomised controlled trials," *Sleep medicine reviews*, 2021, *60*, 101556.

Staehr, Johansen K, "The use of well-being measures in primary health care-the DepCare project," *World Health Organization, Regional Office for Europe: Well-Being Measures in Primary Health Care-the DepCare Project. Geneva: World Health Organization*, 1998.

Steckermeier, Leonie C, "The value of autonomy for the good life. An empirical investigation of autonomy and life satisfaction in Europe," *Social Indicators Research*, 2021, *154* (2), 693–723.

Stevenson, Betsey and Justin Wolfers, "Subjective well-being and income: Is there any evidence of satiation?," *American Economic Review*, 2013, *103* (3), 598–604.

Tenk, Judit, Peter Matrai, Peter Hegyi, Ildiko Rostas, Andras Garami, Imre Szabo, Petra Hartmann, Erika Petervari, Laszlo Czopf, Alizadeh Hussain et al., "Perceived stress correlates with visceral obesity and lipid parameters of the metabolic syndrome: A systematic review and meta-analysis," *Psychoneuroendocrinology*, 2018, *95*, 63–73.

Topp, Christian Winther, Søren Dinesen Østergaard, Susan Søndergaard, and Per Bech, "The WHO-5 Well-Being Index: a systematic review of the literature," *Psychotherapy and psychosomatics*, 2015, *84* (3), 167–176.

Van Parijs, Philippe and Yannick Vanderborght, *Basic income: A radical proposal for a free society and a sane economy*, Harvard University Press, 2017.

Vendrik, Maarten CM, "Adaptation, anticipation and social interaction in happiness: An integrated error-correction approach," *Journal of Public Economics*, 2013, *105*, 131–149.

Vivalt, Eva, Elizabeth Rhodes, Alexander W Bartik, David E Broockman, and Sarah Miller, "The employment effects of a guaranteed income: Experimental evidence from two US states," Technical Report, National Bureau of Economic Research 2024.

Waldinger, Robert and Marc Schulz, *The good life: Lessons from the world's longest scientific study of happiness*, Simon and Schuster, 2023.

West, Stacia and Amy Castro, "Impact of guaranteed income on health, finances, and agency: findings from the Stockton randomized controlled trial," *Journal of Urban Health*, 2023, *100* (2), 227–244.

WHO, World Health Organization, *Comprehensive mental health action plan 2013–2030*, World Health Organization, 2021.

Wu, Dian, Fen Liu, and Shuanglan Huang, "Assessment of the relationship between living alone and the risk of depression based on longitudinal studies: A systematic review and meta-analysis," *Frontiers in Psychiatry*, 2022, *13*, 954857.

Yılmaz Koğar, Esin and Hakan Koğar, "A systematic review and meta-analytic confirmatory factor analysis of the perceived stress scale (PSS-10 and PSS-14)," *Stress and Health*, 2024, *40* (1), e3285.

A Additional Tables, Figures, and Analyses

A.1 Treatment Effects on Mental Health and Wellbeing in Levels

Outcome	Treated	Control	ATE	SE	t-stat	p-N	p-F	n treated	n control
Aggregates									
MHW Index	4.568	4.300	0.268	0.072	3.721	0.000	0.000	107	1477
Mental Health	4.074	3.837	0.237	0.072	3.287	0.001	0.000	107	1477
Purpose in Life	3.003	2.881	0.122	0.067	1.814	0.070	0.100	107	1476
Life Satisfaction	4.471	4.135	0.335	0.063	5.285	0.000	0.000	107	1477
Aggregate components									
WHO-5 Depression	2.916	2.669	0.247	0.078	3.160	0.002	0.000	107	1445
PSS Stress	4.547	4.302	0.246	0.066	3.713	0.000	0.000	107	1470
General life satisfaction	3.783	3.506	0.277	0.065	4.246	0.000	0.000	107	1445
Domain Satisfaction Index	4.446	4.113	0.333	0.064	5.224	0.000	0.000	107	1477
Domain satisfactions									
Health Satisfaction	3.106	2.935	0.171	0.065	2.632	0.001	0.01	107	1477
Sleep Satisfaction	2.637	2.499	0.138	0.056	2.441	0.015	0.016	107	1477
Work Satisfaction	2.638	2.572	0.066	0.056	1.182	0.237	0.262	107	1471
Income Satisfaction	3.222	2.700	0.522	0.066	7.944	0.000	0.000	107	1477
Leisure Satisfaction	2.777	2.574	0.203	0.053	3.859	0.000	0.000	107	1476
Social satisfaction	2.798	2.608	0.190	0.056	3.384	0.001	0.002	107	1476

Table A.1: Average treatment effects on mental health and wellbeing outcomes during the three-year cash transfer program: We report average treatment effects (ATE) in standard deviations on our outcomes in levels (that is without adjustment for baseline differences). Inference is based on robust standard errors (SE), and Neyman (N) and Fisher's exact (F) p-values. We reject the null of no effect for the aggregated mental health and life satisfaction outcomes, the WHO-5 depression scale, the PSS stress scale, the domain satisfaction index, general life satisfaction, and social, health, sleep, income, and leisure satisfaction. We cannot reject the null for work satisfaction and purpose in life. All treatment effects that are statistically significant based on unadjusted p-values remain significant after applying the Benjamini-Hochberg procedure to control the false discovery rate across the m = 56 mental health and wellbeing hypotheses tested in Tables 2, Table A.1, Table A.2, and Table A.3, with the exception of sleep satisfaction 6 months after the program (Table A.2).

A.2 Treatment Effects on Mental Health and Wellbeing after Program

6 Months after Program:

Outcome	Treated	Control	ATE	SE	t-stat	p-N	p-F	n treated	n control
Aggregates									
MHW Index	0.257	-0.074	0.331	0.112	2.950	0.003	0.002	104	1107
Mental Health	0.286	0.028	0.258	0.113	2.284	0.022	0.022	104	1107
Purpose in Life	0.132	-0.175	0.307	0.113	2.717	0.007	0.006	104	1105
Life Satisfaction	0.187	-0.089	0.276	0.117	2.364	0.018	0.016	104	1107
Aggregate components									
WHO-5 Depression	0.348	0.051	0.297	0.119	2.506	0.012	0.012	103	1107
PSS Stress	0.184	0.003	0.181	0.108	1.686	0.092	0.092	104	1104
General Life Satisfaction	0.168	-0.041	0.209	0.116	1.795	0.073	0.070	104	1106
Domain Satisfaction Index	0.173	-0.128	0.301	0.118	2.551	0.011	0.006	104	1107
Domain satisfactions									
Health Satisfaction	-0.069	-0.372	0.303	0.116	2.614	0.009	0.010	104	1107
Sleep Satisfaction†	0.097	-0.136	0.233	0.116	2.009	0.045	0.050	104	1107
Work Satisfaction	-0.078	-0.193	0.115	0.121	0.950	0.342	0.338	104	1087
Income Satisfaction	0.207	0.064	0.143	0.122	1.168	0.243	0.244	104	1104
Leisure Satisfaction	0.373	0.160	0.213	0.118	1.810	0.070	0.056	104	1106
Social Satisfaction	0.132	-0.021	0.153	0.100	1.528	0.127	0.102	104	1106

Table A.2: Average treatment effects on mental health and wellbeing outcomes 6 months after the three-year cash transfer program: We report average treatment effects (ATE) in standard deviations on our outcomes as changes relative to the baseline, see outcome definitions in Table On.7. Inference is based on robust standard errors (SE), and Neyman (N) and Fisher's exact (F) p-values. We reject the null of no effect for the aggregated mental health, purpose in life, and life satisfaction outcomes, the WHO-5 depression scale, the domain satisfaction index, and health and sleep satisfaction. We cannot reject the null for work, social, leisure, and income satisfaction, and for general life satisfaction, and the PSS stress scale. All treatment effects that are statistically significant based on unadjusted p-values remain significant after applying the Benjamini-Hochberg procedure to control the false discovery rate across the m = 56 mental health and wellbeing hypotheses tested in Tables 2, Table A.1, Table A.2, and Table A.3, with the exception of sleep satisfaction 6 months after the program (Table A.2).† Not significant after Benjamini-Hochberg adjustment.

18 Months after Program:

Outcome	Treated	Control	ATE	SE	t-stat	p-N	p-F	n treated	n control
Aggregates									
MHW Index	0.272	-0.091	0.362	0.106	3.416	0.000	0.004	100	1105
Mental Health	0.290	0.012	0.278	0.109	2.558	0.011	0.012	100	1104
Purpose in Life	0.149	-0.165	0.314	0.104	3.014	0.003	0.002	100	1103
Life Satisfaction	0.213	-0.123	0.336	0.128	2.627	0.009	0.014	100	1105
Aggregate components									
WHO-5 Depression	0.280	0.041	0.239	0.112	2.125	0.034	0.03	99	1102
PSS Stress	0.272	-0.027	0.299	0.106	2.812	0.005	0.01	99	1103
General Life Satisfaction	0.201	-0.080	0.281	0.129	2.178	0.029	0.036	99	1105
Domain Satisfaction Index	0.175	-0.151	0.326	0.129	2.517	0.012	0.008	100	1105
Domain satisfactions									
Health Satisfaction	-0.046	-0.340	0.294	0.125	2.359	0.018	0.018	100	1105
Sleep Satisfaction	0.130	-0.147	0.277	0.118	2.339	0.019	0.032	100	1105
Work Satisfaction	0.014	-0.203	0.217	0.129	1.688	0.091	0.088	99	1080
Income Satisfaction	0.162	0.037	0.125	0.126	0.993	0.321	0.302	100	1102
Leisure Satisfaction	0.329	0.143	0.186	0.142	1.311	0.190	0.218	100	1104
Social Satisfaction	0.093	-0.071	0.163	0.110	1.492	0.136	0.120	100	1102

Table A.3: Average treatment effects on mental health and wellbeing outcomes 18 months after the three-year cash transfer program: We report average treatment effects (ATE) in standard deviations on our outcomes as changes relative to the baseline, see outcome definitions in Table On.7. Inference is based on robust standard errors (SE), and Neyman (N) and Fisher's exact (F) p-values. We reject the null of no effect for the aggregated mental health, purpose in life, and life satisfaction outcomes, the WHO-5 depression scale, the the PSS stress scale, general life satisfaction, the domain satisfaction index, and health and sleep satisfaction. We cannot reject the null for work, social, leisure, and income satisfaction. All treatment effects that are statistically significant based on unadjusted p-values remain significant after applying the Benjamini-Hochberg procedure to control the false discovery rate across the m = 56 mental health and wellbeing hypotheses tested in Tables 2, Table A.1, Table A.2, and Table A.3, with the exception of sleep satisfaction 6 months after the program (Table A.2).

A.3 Treatment Effects on Household Finance variables and Time Use in Changes

Outcome	Treated	Control	ATE	SE	t-stat	p-N	p-F	n treated	n control
Financial household									
Donations (6mo)	71.162	-24.719	95.881	20.014	4.791	0.000	0.000	107	1475
Transfers (6mo)	388.759	-50.737	439.497	128.023	3.433	0.001	0.004	107	1474
Savings (monthly)	494.698	39.217	455.481	41.966	10.854	0.000	0.000	107	1473
Debt (stock)	4065.681	4257.961	-192.279	3500.340	-0.055	0.956	0.958	107	1474
Time use, h/w									
Chores	0.397	-0.691	1.087	0.365	2.983	0.003	0.002	107	1465
Education	-0.451	-1.606	1.155	0.691	1.673	0.094	0.104	107	1465
Entertainment	-2.318	-2.423	0.105	1.052	0.100	0.920	0.942	107	1465
Family	1.050	0.050	1.000	0.634	1.576	0.115	0.112	107	1465
Friends	1.322	-0.710	2.031	0.539	3.768	0.000	0.000	107	1465
Partner	2.668	0.802	1.866	1.444	1.292	0.196	0.192	107	1465
Sleep	1.018	-1.520	2.539	0.793	3.202	0.001	0.002	107	1465
Sport	-0.690	-0.167	-0.523	0.296	-1.766	0.077	0.106	107	1465
Volunteering	0.114	-0.153	0.267	0.222	1.202	0.229	0.204	107	1465

Table A.4: We report average treatment effects (ATE) in standard deviations for household finance and time use outcomes in changes. Inference is based on robust standard errors (SE), and Neyman (N) and Fisher's exact (F) p-values.

A.4 Attrition

No. of missing waves	Mental Health	Purpose in life	Life Satisfaction	n control
0	2.049	3.010	4.298	1,124
1	2.032	2.970	4.183	127
2	2.100	3.153	4.436	80
3	2.070	3.020	4.286	70
4	2.134	3.138	4.359	55
5	2.001	2.974	4.286	73
6	2.225	3.075	4.449	51

Table A.5: Average (non-normalized) responses to (i) all mental heath questions, (ii) the purpose in life question, and (iii) all questions regarding life satisfaction at baseline by control participants' number of missing waves later on in the RCT.

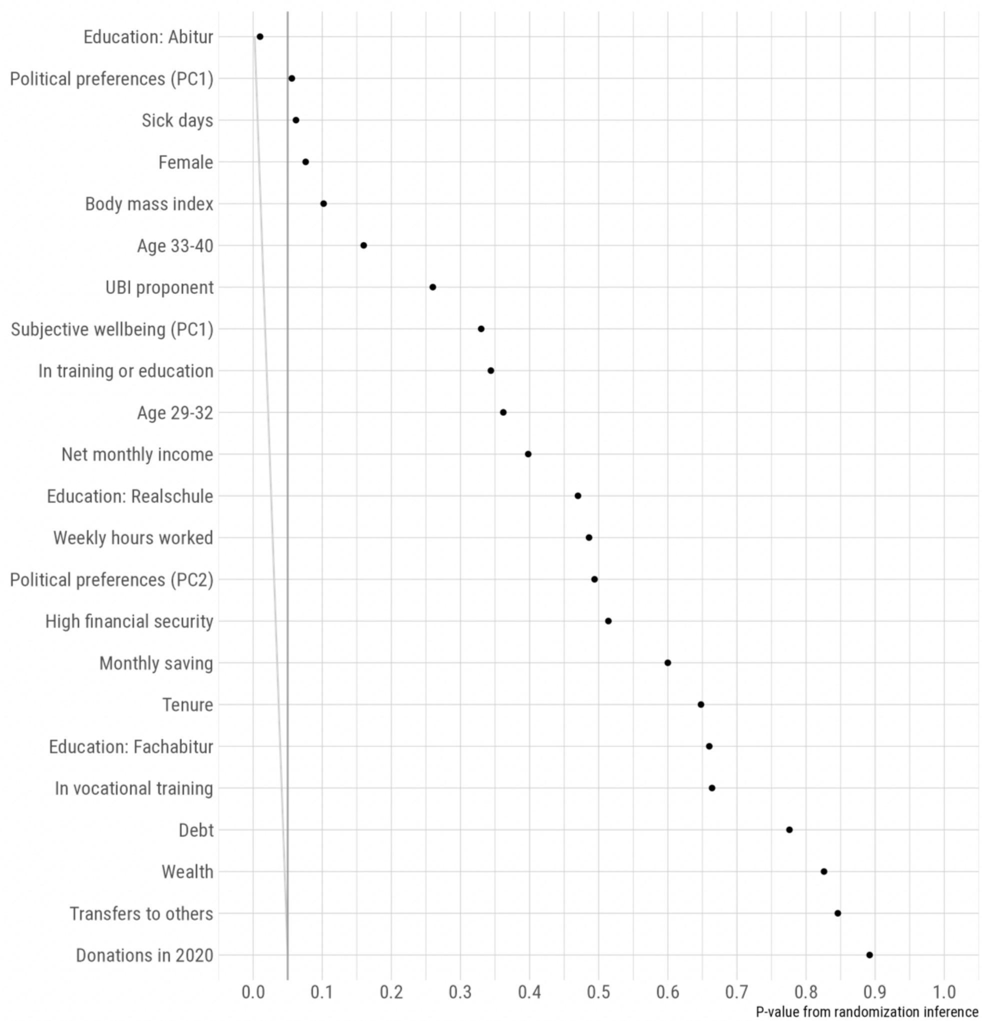

Figure A.1: P-values for differences between treatment and control of baseline covariates, after restricting the sample to observations without any missing waves.

A.5 Sample descriptives

We summarize how our study sample relates to the German population and our study population (the part of the German population that would have been eligible in 2021 to participate in our study) in key demographic variables and wellbeing outcomes at baseline in Table A.6.[10]

	Study sample	Study population	German population
Female	41%	43%	51%
Age	31	31	51
Age between 20-40	100%	100%	34%
Net monthly income (NMI) in EUR	1,934	1,819	2,353
NMI between EUR 1100-2600	100%	100%	55%
Working hours	37.4	38.72	36.35
Non-working	0	6%	33%
Education	11%	4%	2%
Unemployed	4%	2%	4%
Employed	91%	91%	62%
General Life satisfaction	7.23	7.40	7.41
Health satisfaction	8.02	7.68	6.90
Sleep satisfaction	7.14	6.89	6.67
Income satisfaction	6.62	7.24	7.01
Work satisfaction	7.34	6.94	7.00
Leisure satisfaction	6.41	7.08	7.16

Table A.6: Descriptive statistics of the study sample (based on our survey data) relative to study population and German population (based on the SOEP).

A.6 Disappointed control

In Figure A.2, we plot the average main outcomes (non-standardized) in levels that are available also in the SOEP (income satisfaction, leisure satisfaction, general life satisfaction, sleep satisfaction, health satisfaction, and work satisfaction) and compare these average outcomes between our control group (solid black), our treatment group (solid green), our study population (dashed red), and the German population (dashed blue). Please note that the SOEP data is only available until 2023.

The alternative explanation for our treatment effects based on the disappointed-control conjecture would predict that the cash transfers depreciate outcomes of the control rather than improving outcome of the treatment. To identify this, we compare how outcomes evolve over time between our control condition and the study population and the German population We do not find evidence for systemically different directional changes over time between the control and the study population or the control and the German population. General life satisfaction,

[10]We used the SOEP to construct Table A.6. This forces us to restrict attention to the subset of the wellbeing outcomes that were elicited in 2021.

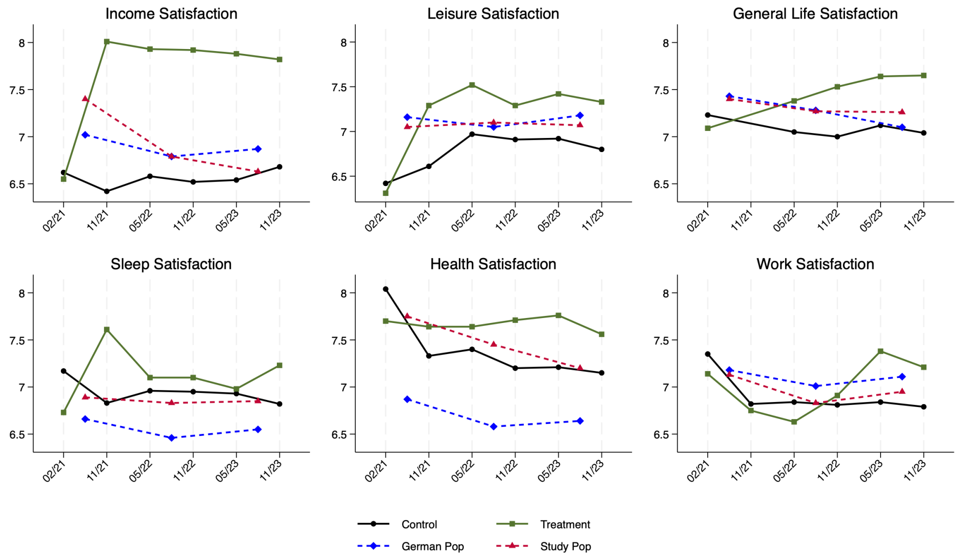

Figure A.2: Average domain-specific satisfaction outcomes over time for the treatment group (solid green), the control group (solid black), the study population (dashed red), and the German population (dashed blue). Each panel displays non-standardized mean satisfaction levels for income satisfaction, leisure satisfaction, general life satisfaction, sleep satisfaction, health satisfaction, and work satisfaction. The study population refers to the subset of the German population that would have been eligible to participate in our study in 2021. Study population and German population data are based on the German Socio-Economic Panel (SOEP), which is available through 2023. The trajectories of the control group, the study population and the German population across all domains, do not provide evidence that control-group disappointment systematically depressed outcomes relative to the broader population.

health satisfaction, and work satisfaction, for instance, decrease over time in the control and, similarly so, in the study population and German population; they however increase over time in the treatment group. Overall, none of these patterns suggest that the wellbeing of our control group changed over time relative to the study population and the German population in ways attributable to control-group disappointment.

Online Appendix for "Cash Transfers, Mental Health, and Agency: Evidence from an RCT in Germany"

by Sandra Bohmann, Susann Fiedler, Maximilian Kasy,
Jürgen Schupp, Frederik Schwerter[11]

March 22, 2026

On Design Details

On.1 Baseline sample

Among the potential participants who satisfied the eligibility criteria after their initial registration, our implementation partner next sampled 20,000 individuals who were invited to participate in a baseline survey. Sampling of these individuals was based on the following criteria. First, the sample was supposed to contain an equal number of proponents and opponents of a universal basic income. Second, potential participants in both of these groups were sampled using a weighted sampling procedure to generate a sample that is close to being representative for the (eligible) German population, and similar across both groups, in terms of age, gender, income, education, employment status, and state ("Bundesland"). The details of this sampling procedure do not affect, in any case, the internal validity or correctness of inference for the study design described below.

On.2 Stratification variables

We used the following 28 stratification variables:

- Subjective wellbeing (PC1): first component of a principle component analysis that is based on an all mental health and wellbeing outcomes that we elicited.

[11] Bohmann: LMU München; sandra.bohmann@lmu.de. Fiedler: WU Wien; susann.fiedler@wu.ac.at. Kasy: Oxford University; maxkasy@gmail.com. Schupp: DIW and FU Berlin; jschupp@diw.de. Schwerter: Frankfurt School of Finance and Management, CESifo, IZA; f.schwerter@fs.de; Corresponding author.

- Age 29-32: a dummy variable =1 if individuals' age is between 29 and 32 years, =0 if individuals' age is below 29 or above 32 years.

- Age 33-40: a dummy variable =1 if individuals' age is between 33 and 40 years, =0 if individuals' age is below 32 years.

- Female: a dummy variable =1 if individuals' gender is female, =0 if individuals' gender is not female.

- German citizen: a dummy variable = 1 if individual is a german citizen, =0 if not =1

- UBI proponent: a dummy variable = 1 if individuals' general attitude towards universal basic income is positive, =0 if it is negative.

- Tenure: a dummy variable =1 if the individual has (at least one) tenured job, =0 if the individual has no tenured job.

- Education: Hauptschule: a dummy variable = 1 if highest education level qualifies for vocational training, =0 if not =1.

- Education: Realschule: a dummy variable = 1 if highest education level qualifies for high school, =0 if not =1.

- Education: Fachabitur: a dummy variable = 1 if highest education level qualifies for vocational academy, =0 if not =1.

- Education: Abitur: a dummy variable = 1 if highest education level qualifies for university, =0 if not =1.
  (Note that the omitted education category is college or more.)

- Net monthly income: net monthly income available to the individual.

- Monthly saving: amount of money saved per month.

- Assets: individuals' total assets.

- Debt: individuals' level of debt.

- High financial security: a dummy variable = 1 if individual states that she could finance herself (with help of others but absent social security benefits) for one year without receiving any income, =0 if not =1.

- Working for money: a dummy variable = 1 if individual works and receives a financial compensation in return, =0 if not =1.

- In training or education: a dummy variable = 1 if individual is in vocational training or receives higher education (undergraduate, graduate, or doctoral level), =0 if not =1.

- In vocational training: a dummy variable = 1 if individual is in vocational training, =0 if not =1.
- Searching work: a dummy variable = 1 if looking for a job, =0 if not looking for a job.
- Sick days: number of workdays missed because of health.
- Weekly hours worked: number of hours worked per week
- Political preferences (PC1): first component of a principle component analysis that is based on an individual's response to how likely (in percent) it is that they vote for either party currently in the German parliament.
- Political preferences (PC2): second component of a principle component analysis that is based on an individual's response to how likely (in percent) it is that they vote for either party currently in the German parliament.
- Body mass index.
- Transfers to others: how much money did the individual give to family members or friends (or others) in 2020.
- Donation in 2020: how much money was donated in 2020.
- Binary gender: a dummy variable =1 if binary gender, =0 if not =1

On.3 Cash transfers were tax free

Each participant's total payment of EUR 43,200 consisted of many small gifts redirected by the implementation partner from individual donors. Because each underlying gift was below the relevant threshold, the payments were tax-free.

On.4 Mental health and wellbeing measures

On.4.1 English translation and discussion

- WHO-5 depression scale (Topp et al., 2015)
  - Over the past 2 weeks...

    ...I have felt cheerful and in good spirits.

    ...I have felt calm and relaxed.

    ...I have felt active and vigorous.

    ...I woke up feeling fresh and rested.

    ...my daily life has been filled with things that interest me.

  - Responses: at no point in time (=0), some of the time (=1), less than half the time (=2), more than half the time (=3), all of the time (=5).

    Note: the original response scale contains a sixth response option “most of the time” (=4) that is not included in our survey because of a software error.

- Perceived stress scale (Cohen et al., 1983), 5 items thereof:
  - In the last month, how often have you...

    ...felt difficulties were piling up so high that you could not overcome them?{−}

    ...felt that you were unable to control the important things in your life?{−}

    ...felt that things were going your way?

    ...felt confident about your ability to handle your personal problems?

    ...felt nervous and ”stressed”? {−}
  - Responses: never (=1), almost never (=2), sometimes (=3), fairly often (=4), very often (=5).
  - To construct outcomes, we use inverted responses for first two items and the final one, which are marked by {−}.

- Purpose in life:
  - Single item, as used in the German Socio-Economic Panel, SOEP, (Richter et al., 2017):
    * Do you feel that what you do in life is meaningful and valuable?
    * Responses are on a 1-11 Likert scale: 1-11, endpoints not at all meaningful and valuable (=1), very much meaningful and valuable (=11).

- Life satisfaction, several scales:
  - General life satisfaction, single item, as used in the German Socio-Economic Panel, SOEP, (Richter et al., 2017):
    * How satisfied are you overall with your life
    * Responses are on a 1-11 Likert scale: 1-11, endpoints very much unsatisfied (=1), very much satisfied (=11).
  - Domain satisfaction, single item for each domain, as used in the German Socio-Economic Panel, SOEP, (Richter et al., 2017):
    * How satisfied are you with your ...

      ...health

      ...sleep

  ...work

  ...income

  ...leisure

  ...social life

  * Responses are on a 1-11 Likert scale: 1-11, endpoints very much unsatisfied (=1), very much satisfied (=11).

On.5 Questions used in explorative analyses

- Perceived autonomy
  - Single item, designed by ourselves.
    * How much do you agree to the following statement? I feel that I can freely determine my life.
    * Response endpoints: do not agree at all (=1), agree entirely (=11).
- Household income
  - Single item
    * When you add up all your income: What is your current net income? Note: Please provide the monthly net amount, i.e., the money that remains after deductions for taxes and social security contributions. Please include regular payments such as housing benefits, child benefits, BAföG (federal educational assistance), alimony, or regular payments from family or friends. Please round the amount to a whole euro amount (without decimal places). Please estimate the monthly amount if you do not know the exact sum.
- Financial security
  - Single item
    * If you suddenly found yourself in an unforeseen situation, could you support yourself for a year without working and without receiving social benefits? Note: For example, through savings or support from family and friends.
- Assets
  - Single item
    * When you add up all your assets (cash and tangible assets including owner-occupied real estate), what do you estimate the total value to be? Note: Please do NOT subtract any debts, mortgages, credits, or loans. Remember, all your information will be stored anonymously and will not allow any conclusions to be drawn about your identity.

- Debt
  - Single item
    - Do you currently have any debts or outstanding loans? Note: Please estimate the total amount of debt if you do not know the exact sum. Remember, all your information will be stored anonymously and will not allow any conclusions to be drawn about your identity.

- Monthly savings
  - Single item
    - Since [Date 6 Months Ago], do you typically have a certain amount left each month that you save or set aside?

- Donations
  - Single item
    - Since [Date 6 Months Ago], have you donated money for social, religious, cultural, or charitable purposes?

- Financial support given to family and friends
  - Single item
    - Since [Date 6 Months Ago], have you have you financially supported relatives, your partner, friends, or acquaintances?

- Consumption
  - Multiple items
    - You will now see some items on which one can, or must, spend money in everyday life. When you think about last month, approximately how many euros did you spend last month on the following items? Note: Please provide an amount in euros. If you did not spend any money on these items, simply enter a 0.
    - Items: housing, energy costs, appliances, daily uses, mobility, leisure activities, apparel (clothes and shoes), and travel.

- Time use

- Multiple items
  * What does your everyday life currently look like in a typical week (Mon–Sun)? Please indicate how many hours per week, on average, you spend on the following activities. Note: Sum up all activities within each category per week. Since activities may overlap, they do NOT need to add up to a total of 168 hours per week.

On.6 Outcome Construction

Outcomes	by wave $t > 0$	across waves $1 \leq t \leq 6$
Mental health		
WHO-5 Depression	$\text{Dep}_{it} = \frac{(dep_{it}-dep_{i0})}{SD(dep_{i0})}$	$\frac{1}{5}\sum_{t=2}^{6} \text{Dep}_{it}$, †
PSS Stress	$\text{Str}_{it} = \frac{(str_{it}-str_{i0})}{SD(str_{i0})}$	$\frac{1}{5}\sum_{t=1,t\neq 2}^{6} \text{Str}_{it}$, †
Mental Health	$\text{MH}_{it} = \frac{(\text{PSS}_{it}+\text{WHO5}_{it})}{2}$	$\frac{1}{4}\sum_{t=3}^{6} \text{MH}_{it}$, ††
Purpose in Life		
Purpose in life	$\text{P}_{it} = \frac{(p_{it}-p_{i0})}{SD(p_{i0})}$	$\frac{1}{6}\sum_{t=1}^{6} \text{P}_{it}$
Life satisfaction		
General Life Satisfaction	$\text{G}_{it} = \frac{(g_{it}-g_{i0})}{SD(g_{i0})}$	$\frac{1}{5}\sum_{t=2}^{6} \text{G}_{it}$, †
Health Satisfaction	$\text{H}_{it} = \frac{(h_{it}-h_{i0})}{SD(h_{i0})}$	$\frac{1}{6}\sum_{t}^{6} \text{H}_{it}$
Sleep Satisfaction	$\text{S}_{it} = \frac{(s_{it}-s_{i0})}{SD(s_{i0})}$	$\frac{1}{6}\sum_{t=1}^{6} \text{S}_{it}$
Income Satisfaction	$\text{I}_{it} = \frac{(i_{it}-i_{i0})}{SD(i_{i0})}$	$\frac{1}{6}\sum_{t=1}^{6} \text{I}_{it}$
Work Satisfaction	$\text{W}_{it} = \frac{(w_{it}-w_{i0})}{SD(w_{i0})}$	$\frac{1}{6}\sum_{t=1}^{6} \text{W}_{it}$
Social Satisfaction	$\text{O}_{it} = \frac{(o_{it}-o_{i0})}{SD(o_{i0})}$	$\frac{1}{6}\sum_{t=1}^{6} \text{O}_{it}$
Leisure Satisfaction	$\text{L}_{it} = \frac{(l_{it}-l_{i0})}{SD(l_{i0})}$	$\frac{1}{6}\sum_{t=1}^{6} \text{L}_{it}$
Domain Satisfaction Index	$\text{D}_{it} = \frac{\text{H}_{it}+\text{S}_{it}+\text{I}_{it}+\text{W}_{it}+\text{O}_{it}+\text{L}_{it}}{6}$	$\frac{1}{6}\sum_{t=1}^{6} \text{D}_{it}$
Life Satisfaction	$\text{Z}_{it} = \frac{\text{D}_{it}+\text{G}_{it}}{2}$	$\frac{1}{5}\sum_{t=2}^{6} \text{Z}_{it}$, ††
Overall index		
MHW Index	$\text{X}_{it} = \frac{\text{MH}_{it}+\text{Z}_{it}+\text{P}_{it}}{3}$	$\frac{1}{4}\sum_{t=3}^{6} \text{X}_{it}$, ††

Table On.7: Outcomes are changes relative to the baseline and are normalized by the standard deviation (SD) at baseline. † The WHO-5 questionnaire and general life satisfaction question were missing in wave $t = 1$, and the PSS questions were missing in wave $t = 2$. †† We compute aggregated outcomes only for waves were all components were elicited.

On.7 Baseline Survey

In Table On.8 in Appendix On.7.1, we list all questions from the baseline survey (without those on labor supply). In Appendix On.7.2 show screenshots of all of these questions in its original version (in German) and an AI-based translation into English.

On.7.1 List of Baseline Variables

Table On.8: Baseline Survey Questions and Response Scales

When were you born?
Month (dropdown); Year (open-ended)
What gender do you identify as?
Female; Male; Diverse; No answer
Do you have German citizenship?
Yes; No
In which federal state do you live?
Dropdown (16 federal states)
What is your postal code?
Open-ended (5-digit code)
How far is it from your home to the center of the nearest major city?
Housing is located in major city center; Under 10 km; 10 to under 25 km; 25 to under 40 km; 40 to under 60 km; 60 km and more
How many people live in your household, including children and yourself?
Open-ended (number)
Do you have a permanent life partner or spouse?
Yes; No; No answer
Do you have children?
Yes; No; No answer
What is your highest general school qualification?
Still in school/training; Junior high school degree; Intermediate school certificate / Middle school diploma; Technical college or general university entrance qualification / High school diploma; No school qualification; No answer
Have you completed vocational training and/or a degree program?
Yes; No
What type of degree is it? (multiple select)
Apprenticeship, skilled worker; Vocational school, commercial school, school of health professions; Technical school, e.g. Master, technical qualification; Civil service training; University of Applied Sciences, Business Academy; University, college degree; Doctorate; Other degree, namely (open-ended)
Are you generally a person willing to take risks or do you try to avoid risks?
1 = not at all risk-taking, . . . , 11 = very risk-taking
How willing were you to give up something that brings you benefit today in order to profit more in the future?
1 = not at all willing to do, . . . , 11 = very willing to do
How willing were you to give for a good cause without expecting anything in return?
1 = not at all willing to do, . . . , 11 = very willing to do
If someone does me a favor, I am willing to return it.
1 = describes me not at all, . . . , 11 = describes me perfectly

Continued on next page

I tend to postpone tasks, even when I know it would be better to do them right away.
  1 = describes me not at all, . . . , 11 = describes me perfectly
If someone puts me in a difficult situation, I will do the same to them.
  1 = describes me not at all, . . . , 11 = describes me perfectly
Unless I'm convinced otherwise, I always assume that other people have only the best intentions.
  1 = describes me not at all, . . . , 11 = describes me perfectly
How likely is it that you will ever vote for this party in a federal election? (CDU/CSU; SPD; FDP; Alliance 90/The Greens; The Left; AfD)
  1 = very unlikely, . . . , 11 = very likely
To what extent do you agree: We should finally find the courage to embrace a strong sense of national identity again.
  1 = strongly disagree, . . . , 5 = strongly agree
To what extent do you agree: What our country needs today is a firm and vigorous assertion of German interests abroad.
  1 = strongly disagree, . . . , 5 = strongly agree
To what extent do you agree: The primary goal of German policy should be to secure for Germany the power and prestige to which it is entitled.
  1 = strongly disagree, . . . , 5 = strongly agree
How would you divide the 100 euros among the following persons? (Person A: extended family member; Person B: organization member; Person C: neighbor; Person D: friend of family member; Person E: colleague; Person F: shared interests/hobbies; Person G: shared religious beliefs; Person H: same age/generation; Person I: shared political views; Person J: shared education level; Person K: yourself)
  0, 5, 10, . . . , 95, 100 euros for each of two recipients per game (must sum to 100)
How would you divide the 100 euros between yourself and an organization? (Pegida; Lichtblick Seniorenhilfe; Sea-Watch e.V.; Bundesvereinigung Lebenshilfe e.V.; Kinderarmut in Deutschland e.V.; Tacheles e.V.; Vier Pfoten in Deutschland; Atmosfair; Parlamentarwatch e.V.)
  0, 5, 10, . . . , 95, 100 euros for each of two recipients per game (must sum to 100)
How satisfied are you with your health?
  1 = very much unsatisfied, . . . , 11 = very much satisfied
How satisfied are you with your sleep?
  1 = very much unsatisfied, . . . , 11 = very much satisfied
How satisfied are you with your work?
  1 = very much unsatisfied, . . . , 11 = very much satisfied
How satisfied are you with your income?
  1 = very much unsatisfied, . . . , 11 = very much satisfied
How satisfied are you with your leisure time?
  1 = very much unsatisfied, . . . , 11 = very much satisfied
How satisfied are you with your social relationships?
  1 = very much unsatisfied, . . . , 11 = very much satisfied
Do you feel that what you do in life is meaningful and valuable?
  1 = not at all meaningful and valuable, . . . , 11 = very much meaningful and valuable
How satisfied are you overall with your life?
  1 = very much unsatisfied, . . . , 11 = very much satisfied
In the last month, how often have you felt difficulties were piling up so high that you could not overcome them?
  1 = never; 2 = seldom; 3 = sometimes; 4 = often; 5 = very often
In the last month, felt that you were unable to control the important things in your life them?
  1 = never; 2 = seldom; 3 = sometimes; 4 = often; 5 = very often
In the last month, how often have you felt that things were going your way?

Continued on next page

	1 = never; 2 = seldom; 3 = sometimes; 4 = often; 5 = very often
In the last month, how often have you felt confident about your ability to handle your personal problems?	
	1 = never; 2 = seldom; 3 = sometimes; 4 = often; 5 = very often
In the last month, how often have you felt nervous and "stressed"?	
	1 = never; 2 = seldom; 3 = sometimes; 4 = often; 5 = very often
How often did you feel nervous and tense? (last 4 weeks)	
	1 = never; 2 = seldom; 3 = sometimes; 4 = often; 5 = very often
Attention check: please select the answer "rarely." (last 4 weeks)	
	1 = never; 2 = seldom; 3 = sometimes; 4 = often; 5 = very often
Over the past two weeks ... I have felt cheerful and in good spirits.	
	1 = never; 2 = seldom; 3 = sometimes; 4 = often; 5 = very often
Over the past two weeks ... I have felt calm and relaxed.	
	1 = never; 2 = seldom; 3 = sometimes; 4 = often; 5 = very often
Over the past two weeks ... I have felt active and vigorous.	
	1 = never; 2 = seldom; 3 = sometimes; 4 = often; 5 = very often
Over the past two weeks ... I woke up feeling fresh and rested.	
	1 = never; 2 = seldom; 3 = sometimes; 4 = often; 5 = very often
Over the past two weeks ... my daily life has been filled with things that interest me.	
	1 = never; 2 = seldom; 3 = sometimes; 4 = often; 5 = very often
How would you describe your current health status?	
	1 = very good; 2 = good; 3 = satisfactory; 4 = fair; 5 = poor; No answer
How often did you have the following everyday complaints in the last two weeks? (Back pain/tension; Headaches; Nausea; Dizziness; Stomach pain; Tinnitus; Irritable bowel syndrome; Respiratory/lung problems)	
	Not at all; On some days; On more than half of the days; (Almost) every day; No answer
How many days in the past twelve months were you unable to work or attend your studies/activity because of illness?	
	Open-ended (number of days); No answer
Have you had COVID-19 in the past twelve months and received medical treatment for it?	
	Yes; No; No answer
Do you currently still have health impairments due to the illness?	
	No; Yes, with minor impairment; Yes, with significant impairment
How tall are you?	
	Open-ended (height in cm); No answer
And how much do you currently weigh?	
	Open-ended (body weight in kg); No answer
Are you currently receiving...? (multiple select)	
	Unemployment benefit 1 (ALG I); Unemployment benefit 2 (ALG II, colloquially Hartz IV); BAföG and/or vocational training allowance; Housing allowance; Child supplement; Parental allowance; Maintenance advance; Benefits under the Asylum Seekers Benefits Act; Short-time working allowance; Wages or salary from employment; Income from self-employment; Income from investments and real estate; Financial gifts from my partner, family or friends; Other social benefits, namely (open-ended); None of the above
If you add up all income: What is your current monthly net income?	
	Open-ended (euros per month); No answer
If you add up all your assets (money and material assets including owner-occupied housing), how much do you estimate the total value to be?	
	No assets; 1 to under 5,000 euros; 5,000 to under 10,000 euros; 10,000 to under 20,000 euros; 20,000 to under 50,000 euros; 50,000 to under 100,000 euros; 100,000 to under 500,000 euros; 500,000 euros and more; No answer

Continued on next page

Table On.8 – *Continued from previous page*

Do you currently have debts or outstanding loans?	
	Yes, approximately (open-ended) euros in total; No; No answer
How much is your monthly payment for principal and interest?	
	Approximately (open-ended) euros per month; No answer
If you suddenly found yourself in an unforeseen situation, could you support yourself for a year without working and without claiming social benefits?	
	Yes; No; No answer
Do you usually have a certain amount left over each month that you can save?	
	Yes, approximately (open-ended) euros per month; No; No answer
Did you donate money in 2020 for social, religious, cultural or charitable purposes?	
	Yes, approximately (open-ended) euros in total; No; No answer
And did you financially support relatives, your partner, friends or acquaintances in 2020?	
	Yes, approximately (open-ended) euros in total; No; No answer
Do you currently live. . . ?	
	For rent; As a subtenant; In owner-occupied housing; Other, namely (open-ended); No answer
How much are your average housing costs (including heating, hot water, electricity, internet, telephone and ancillary costs) in total per month?	
	Open-ended (euros including utilities); No answer
What does your typical week (Mon–Sun) look like currently? Please indicate how many hours per week on average you spend on the following activities.	
	Open-ended (hours per week) for each: Employment; Education and further training; Sports; Time for friends; Time for partnership; Time for family; Volunteer, social or political engagement; Sleep; Information and entertainment; Shopping, household chores, housing, administrative tasks
To what extent do you agree with the following statement? I feel free to decide how to spend my time.	
	1 = strongly disagree, . . . , 5 = strongly agree; No answer
A bat and a ball cost 1.10 euros. The bat costs one euro more than the ball. How much does the ball cost?	
	Open-ended (amount in cents)
5 machines need 5 minutes to produce 5 products. How long do 100 machines need to produce 100 products?	
	Open-ended (minutes)
The water lilies in a pond double in area every day. If the lake is completely covered with water lilies after 48 days, how long did it take to cover half?	
	Open-ended (days)

On.7.2 Screenshots of Baseline Survey

We show below screenshots of the baseline survey (excluding labor supply questions) in the original version (German) in pages On12-On63 and an AI-based translation into English in pages On64-On115.

Herzlich Willkommen zur Basisbefragung für das Pilotprojekt Grundeinkommen!

Das Ausfüllen der Umfrage dauert circa **25 Minuten**.

Einmal getätigte Angaben können NICHT verändert werden.
Es ist deshalb wichtig, alle Beschreibungen genau zu lesen und die Aufgaben sorgfältig zu bearbeiten.

Wir werden Sie nachfolgend zu Ihrer Lebenssituation und persönlichen Einschätzungen befragen.
Versuchen Sie möglichst genau zu antworten. Falls Ihnen Informationen fehlen (wie z. B. die Höhe der Mietkosten, Einnahmen oder Ausgaben), suchen Sie bitte die passenden Unterlagen dazu heraus oder schätzen Sie bestmöglich die Beträge. An anderer Stelle wird es ausschließlich um Ihre persönliche Bewertung gehen. Hier gibt es keine richtigen oder falschen Antworten: Folgen Sie einfach Ihrem Bauchgefühl.

Ihre Angaben werden streng vertraulich behandelt und ausschließlich pseudonymisiert zu Forschungszwecken verwendet. Das bedeutet, dass keine Rückschlüsse auf Ihre Person möglich sind.

Mit Ihrer Teilnahme leisten Sie einen wichtigen Beitrag zur Grundlagenforschung rund ums Bedingungslose Grundeinkommen. Herzlichen Dank!

Bitte füllen Sie die Umfrage bis zum Ende aus. Falls Sie zwischendurch eine Pause benötigen, können Sie durch erneutes Klicken auf den personalisierten Link in Ihrer E-Mail die Bearbeitung fortsetzen.
Bitte nutzen Sie innerhalb der Umfrage nur den "**Weiter**"-Button und nicht den Zurück-Pfeil Ihres Browsers.

Umfrage starten

Zu Beginn einige Fragen zu Ihrer Person.

Wann sind Sie geboren?

Monat

Jahr

Weiter

Welchem Geschlecht fühlen Sie sich zugehörig?

weiblich

männlich

divers

keine Angabe

Weiter

Haben Sie die deutsche Staatsangehörigkeit?

ja

nein

Weiter

In welchem Bundesland leben Sie?

Hinweis: Bitte geben Sie das Bundesland Ihres Hauptwohnsitzes an.

Weiter

Wie lautet Ihre Postleitzahl?

Hinweis: Bitte geben Sie die fünfstellige Postleitzahl Ihres Hauptwohnsitzes an.

Weiter

Wie weit ist es von Ihrem Wohnort bis ins Zentrum der nächsten Großstadt?

Wohnung liegt im Großstadtzentrum

unter 10 km

10 bis unter 25 km

25 bis unter 40 km

40 bis unter 60 km

60 km und mehr

Weiter

Wie viele Personen leben in Ihrem Haushalt, Kinder und Sie selbst eingeschlossen?

Hinweis: Von einem **Haushalt** spricht man, wenn Personen zusammenwohnen UND eine wirtschaftliche Einheit bilden, das heißt Ihre Finanzen weitestgehend zusammenlegen, sich finanziell unterstützen und anfallende Kosten aufteilen.
Wohngemeinschaften (WGs), in denen Bewohner/innen nur die Miet- und Nebenkosten miteinander teilen, gelten nicht als ein gemeinsamer Haushalt.

Weiter

Haben Sie eine/n feste/n Lebenspartner/in oder eine/n Ehepartner/in?

Hinweis: Bitte antworten Sie unabhängig davon, ob sie/er mit Ihnen zusammenwohnt.

ja

nein

keine Angabe

Weiter

Haben Sie Kinder?

Hinweis: Bitte antworten Sie unabhängig davon, ob diese mit Ihnen zusammenwohnen.

ja

nein

keine Angabe

Weiter

Was ist Ihr höchster allgemeiner Schulabschluss?

Hinweis: Bitte geben Sie bei anderen, z. B. ausländischen Abschlüssen, den vergleichbaren Schulabschluss an.

noch in schulischer Ausbildung

Hauptschulabschluss

Realschulabschluss / Mittlere Reife

Fachhochschul- oder Hochschulreife / Abitur

kein Schulabschluss

keine Angabe

Weiter

Haben Sie eine Berufsausbildung und / oder ein Studium abgeschlossen?

ja

nein

Weiter

Um welchen Abschluss handelt es sich?

Hinweis: Bitte wählen Sie alle zutreffenden Angaben aus.

Lehre, Facharbeiter

Berufsfachschule, Handelsschule, Schule des Gesundheitswesens

Fachschule, z.B. Meister-, Technikerabschluss

Beamtenausbildung

Fachhochschule, Berufsakademie

Universitäts-, Hochschulabschluss

Promotion

sonstiger Abschluss, und zwar:

Weiter

On24

Nun folgen einige Fragen zu Ihren Einstellungen & Ansichten.

Wie schätzen Sie sich persönlich ein:
Sind Sie im Allgemeinen ein risikobereiter Mensch oder versuchen Sie, Risiken zu vermeiden?

gar nicht risiko-bereit										sehr risiko-bereit	keine Angabe

Weiter

On25

Wie sehr wären Sie bereit, auf etwas zu verzichten, das heute für Sie Nutzen bringt, um dadurch in Zukunft mehr zu profitieren?

überhaupt nicht bereit, es zu tun										äußerst bereit, es zu tun	keine Angabe

Weiter

On26

Wie sehr wären Sie bereit, für einen guten Zweck zu geben, ohne etwas als Gegenleistung zu erwarten?

über-haupt nicht bereit, es zu tun										äußerst bereit, es zu tun	keine Angabe

Weiter

Wie gut beschreibt diese Aussagen Sie als Person?

Wenn mir jemand einen Gefallen tut, bin ich bereit, ihn zu erwidern.

beschreibt mich überhaupt nicht										beschreibt mich perfekt	keine Angabe

Weiter

Wie gut beschreibt diese Aussagen Sie als Person?

Ich neige dazu, Aufgaben zu verschieben, auch wenn ich weiß, dass es besser wäre, sie gleich zu erledigen.

beschreibt mich überhaupt nicht										beschreibt mich perfekt	keine Angabe

Weiter

Wie gut beschreibt diese Aussagen Sie als Person?

Wenn mich jemand in eine schwierige Lage bringt, werde ich das Gleiche mit ihm/ihr machen.

beschreibt mich überhaupt nicht

beschreibt mich perfekt

keine Angabe

Weiter

Wie gut beschreibt diese Aussagen Sie als Person?

Solange man mich nicht vom Gegenteil überzeugt, gehe ich stets davon aus, dass andere Menschen nur das Beste im Sinn haben.

beschreibt mich überhaupt nicht										beschreibt mich perfekt	keine Angabe

Weiter

Hier sehen Sie eine Liste von politischen Parteien in Deutschland. Geben Sie bitte für jede Partei anhand der Skala an, wie wahrscheinlich es ist, dass Sie diese Partei jemals bei einer Bundestagswahl wählen werden.

	sehr unwahr-scheinlich										sehr wahr-scheinlich	keine Angabe
##v_CDUCSU.												
SPD												
FDP												
Bündnis 90/ Die Grünen												
Die Linke												
AfD												

Weiter

Inwiefern stimmen Sie den folgenden Aussagen zu?

	stimme überhaupt nicht zu	stimme eher nicht zu	teils / teils	stimme eher zu	stimme voll und ganz zu	keine Angabe
Wir sollten endlich wieder Mut zu einem starken Nationalgefühl haben.						
Was unser Land heut braucht, ist ein hartes und energisches Durchsetzen deutscher Interessen gegenüber dem Ausland.						
Das oberste Ziel der deutschen Politik sollte es sein, Deutschland die Macht und Geltung zu verschaffen, die ihm zusteht.						

Weiter

Wir bitten Sie nun, an einer Reihe von hypothetischen Spielen teilzunehmen.

In jedem Spiel (ein Spiel pro Zeile) wird es um verschiedene Personen oder Organisationen gehen. Dazu gehören:

- Sie selbst.
- Personen, die in Deutschland leben und einer bestimmten sozialen Gruppe angehören, z. B. Ihre Nachbar/innen oder Arbeitskolleg/innen.
- Zufällig ausgewählte Personen, die in Deutschland leben.
- Organisationen wie Vier Pfoten in Deutschland oder der Verein Lichtblick Seniorenhilfe.

Weiter

In jedem Spiel werden Sie darum gebeten, Geld aufzuteilen.

In jedem Spiel haben Sie eine **hypothetische Summe von 100 Euro** zur Verfügung, die Sie zwischen zwei verschiedenen Personen und Organisationen verteilen sollen.

Bevor Sie jeweils Ihre Entscheidung treffen, erhalten Sie Informationen über die betroffenen Personen und Organisationen.
Bitte gehen Sie in allen Situationen grundsätzlich davon aus, dass alle betroffenen Personen und Organisationen über das gleiche Einkommen verfügen. Gehen Sie bitte außerdem davon aus, dass die betroffenen Personen und Organisationen nichts über die Identität der verteilenden Person (Sie) erfahren würden.

Sie können in allen Spielen das hypothetische Geld so aufteilen, wie Sie es für richtig erachten.

Weiter

Wie würden Sie die 100 Euro zwischen den Personen aufteilen?

zufällig ausgewählte Person A	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Person aus Ihrer erweiterten Familie (z. B. Cousin)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
zufällig ausgewählte Person B	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
aktuelles oder ehemaliges Mitglied einer Ihrer Vereinigungen (z. B. Schützen- oder Karnevalsverein)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
zufällig ausgewählte Person C	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
jemand, der/die in Ihrer Nachbarschaft wohnt	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
zufällig ausgewählte Person D	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
ein/e Freund/in eines Familienmitglieds (z. B. der beste Freund Ihres Bruders)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
zufällig ausgewählte Person E	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
aktuelle/r oder ehemalige/r Arbeits- oder Schulkolleg/in	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
zufällig ausgewählte Person F	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Person, die Ihre Interessen oder Hobbies teilt (z. B. Fan des gleichen Sportvereins)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
zufällig ausgewählte Person G	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Person mit gleicher Religion	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
zufällig ausgewählte Person H	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Person Ihren Alters / Ihrer Generation	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
zufällig ausgewählte Person I	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Person mit den gleichen politischen Überzeugungen wie Sie (z. B. auch ein/e Linke/r oder ein/e Rechte/r)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
zufällig ausgewählte Person J	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Person mit gleichem Bildungsniveau wie Sie (z. B. auch Abitur oder Hauptschulabschluss)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
zufällig ausgewählte Person K	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Sie selbst	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0

Weiter

Wie würden Sie die 100 Euro zwischen Ihnen und einer Organisation aufteilen?

Sie selbst	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Pegida (Patriotische Europäer gegen die Islamisierung des Abendlandes)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
Sie selbst	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Lichtblick Seniorenhilfe e.V. (Hilfe gegen Altersarmut)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
Sie selbst	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Sea-Watch e.V. (Zivile Seenotrettung von Geflüchteten an Europas Grenzen)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
Sie selbst	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Bundesvereinigung Lebenshilfe e.V. (Hilfe für Menschen mit Behinderung)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
Sie selbst	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Kinderarmut in Deutschland e.V. (Hilfe für benachteiligte Kinder in Deutschland)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
Sie selbst	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Tacheles e.V. (Hilfe für erwerbslose Menschen)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
Sie selbst	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Vier Pfoten in Deutschland (Hilfe für Tiere in Not)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
Sie selbst	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Atmosfair (Unterstützung von Klimaschutzprogrammen)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
Sie selbst	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Parlamentwatch e.V. (Verein, der das Abstimmungsverhalten und Nebeneinnahmen von Politiker/innen veröffentlicht)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0

Weiter

In den nächsten Fragen soll es nun um Ihr persönliches Wohlbefinden und Ihre Gesundheit gehen.

Wie zufrieden sind Sie gegenwärtig mit den folgenden Bereichen Ihres Lebens?

	ganz und gar unzufrieden										voll-kommen zufrieden	keine Angabe
Mit Ihrer Gesundheit?												
Mit Ihrem Schlaf?												
Mit Ihrer Arbeit?												
Mit Ihrem persönlichen Einkommen?												
Mit Ihrer Freizeit?												
Mit Ihren sozialen Beziehungen?												

Weiter

Haben Sie das Gefühl, dass das, was Sie in Ihrem Leben machen, wertvoll und nützlich ist?

überhaupt nicht wertvoll und nützlich										vollkommen wertvoll und nützlich	keine Angabe

Weiter

Bitte denken Sie einmal an die letzten vier Wochen.

Wie oft ...

	sehr selten / nie	selten	manchmal	häufig	sehr häufig	keine Angabe
... hatten Sie das Gefühl, dass sich die Probleme so aufgestaut haben, dass Sie diese nicht mehr bewältigen können?						
... hatten Sie das Gefühl, wichtige Dinge in Ihrem Leben nicht beeinflussen zu können?						
... hatten Sie das Gefühl, dass sich die Dinge nach Ihren Vorstellungen entwickeln?						
... hatten Sie das Gefühl, dass Sie mit persönlichen Problemen gut umgehen können?						
... fühlten Sie sich gehetzt und unter Zeitdruck?						
... fühlten Sie sich nervös und angespannt?						
... mit dieser Teilfrage wollen wir Ihre Aufmerksamkeit testen. Bitte klicken Sie die Antwort "selten" an.						

Weiter

Die folgenden Aussagen betreffen Ihr Wohlbefinden in den letzten zwei Wochen.
Bitte geben Sie jeweils an, welche Aussage am besten beschreibt, wie Sie sich in den letzten zwei Wochen gefühlt haben.

In den letzten zwei Wochen ...

	zu keinem Zeitpunkt	ab und zu	etwas weniger als die Hälfte der Zeit	etwas mehr als die Hälfte der Zeit	die ganze Zeit	keine Angabe
... war ich froh und guter Laune.						
... habe ich mich ruhig und entspannt gefühlt.						
... habe ich mich energisch und aktiv gefühlt.						
... habe ich mich beim Aufwachen frisch und ausgeruht gefühlt.						
... war mein Alltag voller Dinge, die mich interessieren.						

Weiter

Wie zufrieden sind Sie aktuell im Großen und Ganzen mit Ihrem Leben?

ganz und gar unzufrieden										vollkommen zufrieden	keine Angabe

Weiter

Wie würden Sie gegenwärtig Ihren Gesundheitszustand beschreiben?

sehr gut

gut

zufriedenstellend

weniger gut

schlecht

keine Angabe

Weiter

Wie häufig hatten Sie in den vergangenen zwei Wochen folgende Alltagsbeschwerden?

	überhaupt nicht	an einzelnen Tagen	an mehr als der Hälfte der Tage	(fast) jeden Tag	keine Angabe
Rückenschmerzen / Verspannungen					
Kopfschmerzen					
Übelkeit					
Schwindelgefühle					
Magenschmerzen					
Tinnitus					
Reizdarm					
Atemwegs- / Lungenprobleme					

Weiter

Wie viele Tage haben Sie in den vergangenen zwölf Monaten wegen Krankheit nicht gearbeitet, bzw. konnten Ihrem Studium / Ihrer Tätigkeit nicht nachgehen?

Hinweis: Bitte geben Sie alle Tage an, nicht nur die, für die Sie eine ärztliche Arbeitsunfähigkeitsbescheinigung erhalten haben.
Wenn Sie die exakte Anzahl der Tage nicht wissen, schätzen Sie bitte.

keine Angabe

Weiter

Waren Sie in den letzten zwölf Monaten an Corona erkrankt und deshalb in ärztlicher Behandlung?

ja

nein

keine Angabe

Weiter

Haben Sie derzeit immer noch gesundheitliche Beeinträchtigungen aufgrund der Erkrankung?

nein

ja, mit geringer Beeinträchtigung

ja, mit erheblicher Beeinträchtigung

Weiter

Wie groß sind Sie?

Körpergröße in cm

keine Angabe

Weiter

Und wie viel wiegen Sie zurzeit?

Körpergewicht in kg

keine Angabe

Weiter

Nun folgen einige Fragen zu Ihrer finanziellen Situation und Ihrem Umgang mit Geld.

Erhalten Sie derzeit ...?

Hinweis: Bitte wählen Sie alle zutreffenden Angaben aus.

Arbeitslosengeld 1 (ALG I)

Arbeitslosengeld 2 (ALG II, umgangssprachlich Hartz IV)

Bafög und oder Berufsausbildungsbeihilfe

Wohngeld

Kinderzuschlag

Elterngeld

Unterhaltsvorschuss

Leistungen gemäß des Asylbewerberleistungsgesetzes

Kurzarbeitergeld

Lohn oder Gehalt aus nicht-selbständiger Tätigkeit

Lohn aus selbstständiger Tätigkeit

Einkünfte aus Kapitalanlagen und Immobilien

Finanzielle Zuwendungen von meinem/meiner Partner/in, Familie oder Freund/innen

andere Sozialleistungen, und zwar:

nichts davon

Weiter

Wenn Sie alle Einkünfte zusammenrechnen: Wie hoch ist Ihr aktuelles monatliches Nettoeinkommen?

Hinweis: Bitte geben Sie den monatlichen Nettobetrag an, also das Geld, das nach Abzug von Steuern und Sozialabgaben übrig bleibt. Regelmäßige Zahlungen wie Wohngeld, Kindergeld, BAföG, Unterhaltszahlungen oder regelmäßige Zahlungen von Familie oder Freund/innen rechnen Sie bitte dazu.

Bitte geben Sie den Betrag gerundet auf einen ganzen Eurobetrag (ohne Nachkommazahlen) an. Bitte schätzen Sie den monatlichen Betrag, falls Ihnen die genaue Summe nicht bekannt ist.

Euro im Monat

keine Angabe

Weiter

Wenn Sie Ihr gesamtes Vermögen zusammenrechnen (Geld- und Sachvermögen einschließlich selbstgenutztem Wohneigentum), wie hoch schätzen Sie den Gesamtwert?

Hinweis: Bitte ziehen Sie eventuelle Schulden, aufgenommene Hypotheken, Kredite oder Darlehen **NICHT** ab.

Zur Erinnerung, alle Ihre Angaben werden anonymisiert abgespeichert und erlauben keine Rückschlüsse auf Ihre Person.

- keinerlei Vermögen
- 1 bis unter 5.000 Euro
- 5.000 bis unter 10.000 Euro
- 10.000 bis unter 20.000 Euro
- 20.000 bis unter 50.000 Euro
- 50.0000 bis unter 100.000 Euro
- 100.000 bis unter 500.000 Euro
- 500.000 Euro und mehr
- keine Angabe

Weiter

Haben Sie derzeit Schulden oder laufende Kredite?

Hinweis: Bitte schätzen Sie den gesamten Schuldenbetrag, falls Ihnen die genaue Summe nicht bekannt ist.

Zur Erinnerung, alle Ihre Angaben werden anonymisiert abgespeichert und erlauben keine Rückschlüsse auf Ihre Person.

ja, und zwar insgesamt etwa: ______ Euro

nein

keine Angabe

Weiter

Wie hoch ist Ihre monatliche Belastung für Tilgung und Zinsen?

monatlich in etwa: ____ Euro

keine Angabe

Weiter

Wenn Sie plötzlich in eine unvorhergesehene Situation geraten würden, könnten Sie ein Jahr lang Ihren Unterhalt bestreiten, ohne einer Arbeit nachzugehen und ohne Sozialleistungen in Anspruch zu nehmen?

Hinweis: Zum Beispiel durch Erspartes oder Unterstützung von Familie und Freund/innen.

ja

nein

keine Angabe

Weiter

Bleibt Ihnen in der Regel monatlich ein gewisser Betrag, den Sie sparen oder zurücklegen?

Hinweis: Dabei kann es sich um regelmäßige Spareinlagen zur Vermögensbildung handeln, wie zum Beispiel: Banksparpläne, private Rentenverträge oder Bausparverträge.
Es geht aber auch um Bargeldsparen, zum Beispiel für größere Anschaffungen oder Notlagen.

ja, und zwar monatlich etwa: ______ Euro

nein

keine Angabe

Weiter

Haben Sie im Jahr 2020 für soziale, kirchliche, kulturelle oder gemeinnützige Zwecke Geld gespendet?

Hinweis: Bitte schätzen Sie den gesamten Jahresbetrag, falls Ihnen der genaue Betrag nicht bekannt ist.

ja, und zwar insgesamt etwa: ____ Euro

nein

keine Angabe

Weiter

Und haben Sie im Jahr 2020 Verwandte, Ihre/n Partner/in, Freund/innen oder Bekannte finanziell unterstützt?

Hinweis: Bitte schätzen Sie den gesamten Jahresbetrag, falls Ihnen der genaue Betrag nicht bekannt ist.

ja, und zwar mit insgesamt etwa: ______ Euro

nein

keine Angabe

Weiter

Wohnen Sie aktuell ... ?

zur Miete

zur Untermiete

im Wohneigentum

sonstiges, und zwar:

keine Angabe

Weiter

Wie hoch sind Ihre durchschnittlichen Wohnkosten (inklusive Heizkosten, Warmwasser, Strom, Internet, Telefon und Nebenkosten) insgesamt pro Monat?

Hinweis: Sollten Sie aktuell mietfrei wohnen, z. B. im Haushalt der Eltern, des Partners, der Partnerin oder Ähnliches, geben Sie bitte 0 an.

Euro (warm)

keine Angabe

Weiter

Wie sieht gegenwärtig Ihr Alltag in einer typischen Woche (Mo-So) aus?
Bitte geben Sie an, wie viel Stunden pro Woche Sie im Durchschnitt mit den folgenden Aktivitäten verbringen.

Hinweis: Addieren Sie dabei alle Aktivitäten einer Kategorie pro Woche auf. Da Aktivitäten sich überschneiden können, müssen sie sich **NICHT** auf den Gesamtwert von 168 Wochenstunden aufaddieren.

	Stunden pro Woche (Mo-So)
Erwerbstätigkeit (einschl. möglicher Überstunden und Wegezeiten)	
Aus- und Weiterbildung, Lernen (z. B. Kurse, Schule, Studium, Promotion)	
Sport (z. B. Vereinssport, Fitnessstudio, Yoga)	
Zeit für Freunde (z. B. gemeinsame Aktivitäten, Kommunikation)	
Zeit für Partnerschaft (z. B. gemeinsame Aktivitäten, Kommunikation)	
Zeit für Familie (z. B. gemeinsame Aktivitäten, Kommunikation)	
Ehrenamtliches, soziales oder politisches Engagement (z. B. Vereinsarbeit)	
Schlaf	
Information und Unterhaltung (z. B. Fernsehen, Kino, Theater, Spiele, Social Media)	
Besorgungen, Hausarbeit, Wohnung, Vewaltungsaufgaben (z. B. Rechnungen überweisen)	

Weiter

Inwiefern stimmen Sie der folgenden Aussage zu?

Ich habe das Gefühl, frei über meine Zeit bestimmen zu können.

stimme überhaupt nicht zu				stimme voll und ganz zu	keine Angabe

Weiter

Jetzt folgen drei Aufgaben, die in ihrer Schwierigkeit variieren. Versuchen Sie bitte so viele wie möglich davon zu lösen.

Ein Schläger und ein Ball kosten 1,10 Euro. Der Schläger kostet einen Euro mehr als der Ball.
Wie viel kostet der Ball?

Angabe in Cent:

5 Maschinen benötigen für die Produktion von 5 Produkten 5 Minuten.
Wie lange benötigen 100 Maschinen für die Produktion von 100 Produkten?

Die Seerosen in einem Teich verdoppeln ihre Fläche jeden Tag.
Wenn der See nach 48 Tagen komplett mit Seerosen bedeckt ist, wie lange hat es gebraucht, bis er zur Hälfte bedeckt war?

Weiter

Welcome to the baseline survey for the Pilot Project Basic Income!

Filling out the survey takes approximately 25 minutes.

Once entered, answers cannot be changed.

It is therefore important to read all descriptions carefully and complete the tasks carefully.
We will ask you about your life situation a n d personal assessments.

Try to answer as accurately as possible. If you lack information (such as the amount of rent, income or expenses), please find the relevant documents or estimate the amounts as best as possible. In other places, it will be exclusively about your personal assessment. There are no right or wrong answers here: Just follow your gut feeling.

Your information will be treated strictly confidentially and used exclusively in anonymized form for research purposes. This means that no conclusions about your person are possible.

By participating, you make an important contribution to basic research on unconditional

Basic Income. Thank you!

Please complete the survey until the end. If you need a break, you can continue working on it by clicking on the personalized link in your email again. Please use only the 'Next' button within the survey and not the back arrow of your browser.

Umfrage starten

To begin, a few questions about yourself.

When were you born?

Month

Year

Weiter

What gender do you identify as?

female

male

diverse

no answer

Weiter

Do you have German citizenship?

Yes

No

Weiter

In which federal state do you live?

Note: Please specify the federal state of your primary residence.

Weiter

What is your postal code?

Note: Please enter the five-digit postal code of your primary residence.

Weiter

How far is it from your home to the center of the nearest major city?

Housing is located in major city center

under 10 km

10 to under 25 km

25 to under 40 km

40 to under 60 km

60 km and more

Weiter

How many people live in your household, including children and yourself?

Note: A household is when people live together AND form an economic unit, meaning you largely pool your finances,
to financially support and share incurred costs.
Shared apartments (WGs) where residents only share rent and ancillary costs are not considered a joint household.

Weiter

Do you have a permanent life partner or spouse?

Note: Please answer regardless of whether they live with you.

yes

no

no answer

Weiter

Do you have children?

Note: Please answer regardless of whether they live with you.

yes

no

no answer

Weiter

What is your highest general school qualification?

Note: For other qualifications, such as foreign degrees, please specify the comparable school qualification.

still in school/training

Junior high school degree

Intermediate school certificate / Middle school diploma

Technical college or general university entrance qualification / High school diploma

no school qualification

no answer

Weiter

Have you completed vocational training and/or a degree program?

yes

nein

Weiter

What type of degree is it?

Note: Please select all applicable answers.

- Apprenticeship, skilled worker
- Vocational school, commercial school, school of health professions
- Technical school, e.g. Master, technical qualification
- Civil service training
- University of Applied Sciences, Business Academy
- University, college degree
- Doctorate
- other degree, namely:

Weiter

Now come some questions about your attitudes and views.

How would you rate yourself:

Are you generally a person willing to take risks or do you try to avoid risks?

not at all risk-taking										very risk-taking	no answer

Weiter

How willing were you to give up something that brings you benefit today in order to profit more in the future?

not at all willing to do										very willing to do	no answer

Weiter

How willing were you to give for a good cause without expecting anything in return?

not at all willing to do										very willing to do	no answer

Weiter

How well do these statements describe you as a person?

If someone does me a favor, I am willing to return it.

describes me not at all										describes me perfectly	no answer

Weiter

How well do these statements describe you as a person?

I tend to postpone tasks, even when I know it would be better to do them right away.

describes me not at all										describes me perfectly	no answer

Weiter

How well do these statements describe you as a person?

If someone puts me in a difficult situation, I will do the same to them.

describes me not at all										describes me perfectly	no answer

Weiter

How well do these statements describe you as a person?

Unless I'm convinced otherwise, I always assume that other people have only the best intentions.

describes me not at all										describes me perfectly	no answer

Weiter

Here you see a list of political parties in Germany. Please indicate on the scale for each party how likely it is that
that you will ever vote for this party in a federal election.

	very unlikely										very likely	no answer
CDU/CSU												
SPD												
FDP												
Alliance 90/The Greens												
The Left												
AfD												

Weiter

On84

To what extent do you agree with the following statements?

	strongly disagree	somewhat disagree	I'm not sure	somewhat agree	strongly sagree	no answer
We should finally find the courage to embrace a strong sense of national identity again						
What our country needs today is a firm and vigorous assertion of German interests abroad.						
The primary goal of German policy should be to secure for Germany the power and prestige to which it is entitled.						

Weiter

We now ask you to participate in a series of hypothetical games.

In each game (one game per row), it will be about different persons or organizations. These include:

- You yourself.
- Persons living in Germany and belonging to a particular social group, e.g. your neighbors or colleagues.
- Randomly selected persons living in Germany.
- Organizations like Four Paws in Germany or the Light Beam Senior Help Association.

Weiter

In each game you will be asked to distribute money.

In each game you have a hypothetical sum of 100 euros at your disposal, which you will distribute between two different persons and should distribute to organizations.

Before you make your decision, you will receive information about the affected persons and organizations.
Please assume in all situations that all affected persons and organizations have the same income
be available. Please also assume that the affected persons and organizations know nothing about the identity of the distributing person (you) would experience.

In all games, you can distribute the hypothetical money as you see fit.

Weiter

How would you divide the 100 euros among the following persons?
Randomly selected
Person A
A member of your extended family (e.g., your cousin)
0 5 10 15 20 25 30 35 40 45 50 55 60 65 70 75 80 85 90 95 100
100 95 90 85 80 75 70 65 60 55 50 45 40 35 30 25 20 15 10 5 0
Randomly selected
Person B
A member of one of your past or current organizations (local church, leisure club or association, etc.)
0 5 10 15 20 25 30 35 40 45 50 55 60 65 70 75 80 85 90 95 100
100 95 90 85 80 75 70 65 60 55 50 45 40 35 30 25 20 15 10 5 0
Randomly selected
Person C
Someone who lives in your local neighborhood
0 5 10 15 20 25 30 35 40 45 50 55 60 65 70 75 80 85 90 95 100
100 95 90 85 80 75 70 65 60 55 50 45 40 35 30 25 20 15 10 5 0
Randomly selected
Person D
Someone who is a friend of a family member (e.g., your sibling's closest friend)
0 5 10 15 20 25 30 35 40 45 50 55 60 65 70 75 80 85 90 95 100
100 95 90 85 80 75 70 65 60 55 50 45 40 35 30 25 20 15 10 5 0
Randomly selected
Person E
A former or current colleague at work or school
0 5 10 15 20 25 30 35 40 45 50 55 60 65 70 75 80 85 90 95 100
100 95 90 85 80 75 70 65 60 55 50 45 40 35 30 25 20 15 10 5 0
Randomly selected
Person F
Someone who shares your interests or hobbies (e.g., a fellow fan of the same sports team, or a fellow runner)
0 5 10 15 20 25 30 35 40 45 50 55 60 65 70 75 80 85 90 95 100
100 95 90 85 80 75 70 65 60 55 50 45 40 35 30 25 20 15 10 5 0
Randomly selected
Person G
Someone who shares your religious beliefs
0 5 10 15 20 25 30 35 40 45 50 55 60 65 70 75 80 85 90 95 100
100 95 90 85 80 75 70 65 60 55 50 45 40 35 30 25 20 15 10 5 0
Randomly selected
Person H
Someone of your same age/ generation
0 5 10 15 20 25 30 35 40 45 50 55 60 65 70 75 80 85 90 95 100
100 95 90 85 80 75 70 65 60 55 50 45 40 35 30 25 20 15 10 5 0
Randomly selected
Person I
Someone who shares your political views (e.g., a fellow left-winger, or a fellow right-winger, etc.)
0 5 10 15 20 25 30 35 40 45 50 55 60 65 70 75 80 85 90 95 100
100 95 90 85 80 75 70 65 60 55 50 45 40 35 30 25 20 15 10 5 0
Randomly selected
Person J
Someone who shares your level of education
0 5 10 15 20 25 30 35 40 45 50 55 60 65 70 75 80 85 90 95 100
100 95 90 85 80 75 70 65 60 55 50 45 40 35 30 25 20 15 10 5 0
Randomly selected
Person K
Yourself
0 5 10 15 20 25 30 35 40 45 50 55 60 65 70 75 80 85 90 95 100
100 95 90 85 80 75 70 65 60 55 50 45 40 35 30 25 20 15 10 5 0
Weiter

How would you divide the 100 euros between yourself and an organization?

Yourself	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Pegida (Patriotische Europäer gegen die Islamisierung des Abendlandes)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
Yourself	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Lichtblick Seniorenhilfe e.V. (Hilfe gegen Altersarmut)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
Yourself	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Sea-Watch e.V. (Zivile Seenotrettung von Geflüchteten an Europas Grenzen)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
Yourself	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Bundesvereinigung Lebenshilfe e.V. (Hilfe für Menschen mit Behinderung)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
Yourself	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Kinderarmut in Deutschland e.V. (Hilfe für benachteiligte Kinder in Deutschland)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
Yourself	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Tacheles e.V. (Hilfe für erwerbslose Menschen)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
Yourself	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Vier Pfoten in Deutschland (Hilfe für Tiere in Not)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
Yourself	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Atmosfair (Unterstützung von Klimaschutzprogrammen)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0
Yourself	0	5	10	15	20	25	30	35	40	45	50	55	60	65	70	75	80	85	90	95	100
Parlamentwatch e.V. (Verein, der das Abstimmungs-verhalten und Nebeneinnahmen von Politiker/innen veröffentlicht)	100	95	90	85	80	75	70	65	60	55	50	45	40	35	30	25	20	15	10	5	0

Weiter

The next questions are about your personal well-being and health.

How satisfied are you currently with the following areas of your life?

	not at all satisfied										entirely satisfied
With your health?											
With your sleep?											
With your work?											
With your personal income?											
With your leisure time?											
With your social relationships?											

Weiter

Do you feel that what you do in your life is valuable and useful?

not at all valuable and useful										entirely valuable and useful	no answer

Weiter

Please think back to the last four weeks.

How often...

	never	seldom	sometimes	often	very often	no answer
…did you feel like the problems had piled up to the point where you could no longer handle them?						
…did you feel that you had no control over important aspects of your life?						
…did you feel that things were going the way you wanted them to?						
…did you feel that you were able to handle personal problems well?						
…did you feel rushed and under time pressure?						
…did you feel nervous and tense?						
… with this question, we want to test your attention. Please select the answer "rarely."						

Weiter

The following statements refer to how you have been feeling over the past two weeks.
Please indicate which statement best describes how you have been feeling over the past two weeks.

Over the past two weeks ...

	never	seldom	sometimes	often	very often	no answer
… I felt happy and in a good mood.						
… I felt calm and relaxed.						
… I felt energetic and active.						
… I felt refreshed and well-rested when I woke up.						
… my day was filled with things that interest me.						

Weiter

How satisfied are you currently overall with your life?

not at all satisfied										entirely satisfied	no answer

Weiter

How would you describe your current health status?

very good

good

satisfactory

fair

poor

no answer

Weiter

How often did you have the following everyday complaints in the last two weeks?

	not at all	on some days	on more than half of the days	(almost) every day	no answer
Back pain / tension					
Headaches					
Nausea					
Dizziness					
Stomach pain					
Tinnitus					
Irritable bowel syndrome					
Respiratory/lung problems					

Weiter

How many days in the past twelve months were you unable to work or attend your studies / activity because of illness, or were unable to attend?

Note: Please list all days, not just those for which you received a medical certificate of incapacity to work.
If you don't know the exact number of days, please estimate.

no answer

Weiter

Have you had COVID-19 in the past twelve months and received medical treatment for it?

yes

no

no answer

Weiter

Do you currently still have health impairments due to the illness?

no

yes, with minor impairment

yes, with significant impairment

Weiter

How tall are you?

Height in cm

no answer

Weiter

And how much do you currently weigh?

Body weight in kg

no answer

Weiter

The following are some questions about your financial situation and how you manage money.

Are you currently receiving...?

Note: Please select all applicable answers.

- Unemployment benefit 1 (ALG I)
- Unemployment benefit 2 (ALG II, colloquially Hartz IV)
- BAföG and/or vocational training allowance
- Housing allowance
- Child supplement
- Parental allowance
- Maintenance advance
- Benefits under the Asylum Seekers Benefits Act
- Short-time working allowance
- Wages or salary from employment
- Income from self-employment
- Income from investments and real estate
- Financial gifts from my partner, family or friends
- other social benefits, namely:
- none of the above

Weiter

If you add up all income: What is your current monthly net income?

Note: Please specify the monthly net amount, that is the money left after deducting taxes and social contributions. Regular payments such as housing allowance, child allowance, BAföG, maintenance payments or regular payments from family or friends, please include.

Please give the amount rounded to a whole euro (without decimal places). Please estimate the monthly amount if you do not have the exact sum is known.

euros per month

no answer

Weiter

If you add up all your assets (money and material assets including owner-occupied housing), how much do you estimate the total value to be?

Note: Please do not subtract any debts, mortgages, loans or credits taken out.

As a reminder, all your information will be stored anonymously and allow no conclusions about your person.

no assets

1 to under 5,000 euros

5,000 to under 10,000 euros

10,000 to under 20,000 euros

20,000 to under 50,000 euros

50,000 to under 100,000 euros

100,000 to under 500,000 euros

500,000 euros and more

no answer

Weiter

Do you currently have debts or outstanding loans?

Note: Please estimate the total debt amount if you don't know the exact sum.

As a reminder, all your information will be stored anonymously and allow no conclusions about your person.

yes, approximately: euros in total

no

no answer

Weiter

How much is your monthly payment for principal and interest?

approximately: euros per month

no answer

Weiter

If you suddenly found yourself in an unforeseen situation, could you support yourself for a year without working and without claiming social benefits?

Note: For example through savings or support from family and friends.

yes

no

no answer

Weiter

Do you usually have a certain amount left over each month that you can save?

Note: This can be regular savings deposits for wealth building, such as: bank savings plans, private pension policies or home savings contracts.
This also includes saving cash, for example for larger purchases or emergencies.

yes, approximately: euros per month

no

no answer

Weiter

Did you donate money in 2020 for social, religious, cultural or charitable purposes?

Note: Please estimate the total annual amount if you do not know the exact amount.

yes, approximately: euros in total

no

no answer

Weiter

And did you financially support relatives, your partner, friends or acquaintances in 2020?

Note: Please estimate the total annual amount if you do not know the exact amount.

yes, approximately: euros in total

no

no answer

Weiter

Do you currently live...?

for rent

as a subtenant

in owner-occupied housing

other, namely:

no answer

Weiter

How much are your average housing costs (including heating, hot water, electricity, internet, telephone and ancillary costs) in total per month?

Note: If you currently live rent-free, e.g. in the household of your parents, your partner or similar, please enter 0.

euros (including utilities)

no answer

Weiter

What does your typical week (Mon-Sun) look like currently?
Please indicate how many hours per week on average you spend on the following activities.

Note: Add up all activities in each category per week. Since activities can overlap, they do NOT need to add up to the total of 168 weekly hours.

	Hours per week (Mon-Sun)
Employment (including possible overtime and travel time)	
Education and further training, learning (e.g. courses, school, studies, doctorate)	
Sports (e.g. club sports, gym, yoga)	
Time for friends (e.g. joint activities, communication)	
Time for partnership (e.g. joint activities, communication)	
Time for family (e.g. joint activities, communication)	
Volunteer, social or political engagement (e.g. club work)	
sleep	
Information and entertainment (e.g. television, cinema, theater, games, social media)	
Shopping, household chores, housing, administrative tasks (e.g. transferring bills)	

Weiter

To what extent do you agree with the following statement?

I feel free to decide how to spend my time.

strongly disagree				strongly agree	no answer

Weiter

The following are three tasks that vary in difficulty. Please try to solve as many as you can.

A bat and a ball cost 1.10 euros. The bat costs one euro more than the ball.
How much does the ball cost?

Amount in cents:

5 machines need 5 minutes to produce 5 products.
How long do 100 machines need to produce 100 products?

The water lilies in a pond double in area every day.
If the lake is completely covered with water lilies after 48 days, how long did it take to cover half?

Weiter